جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : عفاف احمد السيد عيسوى
المحافظة : القليوبيه **الحي / المدينة :** بنها **العنوان :** --
رقم البطاقة : 27206260102469 **رقم التليفون :** 01281289063 **تاريخ تسجيل الإستمارة :** 19-10-2026
العمر(تاريخ الميلاد) : 26-06-1972 **المهنة :** بدون عمل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام اشعة مقطعية مخ مع صبغة [10851] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس
وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

أ.د/ ---------- **أ.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: علاف احمد السيد عيسوي
National ID: 27206260102469

GUSTAVE ROUSSY

تقرير طبي

السيد

تحية طيبة وبعد،،

بشان تقرير طبي للمريض	ايماء للطلب المقدم من
رقم طبي:	يقسم:

تتشرف بان توضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :	تاريخ وساعة الخروج :
yyyy-mm-dd	PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالمخ و تحتاج الي Contrast-enhanced CT of the brain.

التاريخ :	توقيع :
05-07-2026	
	اطبيب المعالج :


جمهورية مصر العربية
بطاقة تحقيق الشخصية
عطاف
KE5654972


دبلوم تجاره
الشي
منزوجة
٩

GUSTAVE ROUSSY

7-8-2025

elevate private equity

MDT

Patient Name	عفاف احمد السيد	Age	52
Sex	Female	Special habits	
Family History		Co-morbidity	
Surgical History		Telephone	01281289063
Menopausal Status	POSTPREMENOPAUSAL	Marital Status	
MEDICAL ID	108/2025	ID	27206260102469

Pathology Bx: IDC GII
ER:0
PR:0
Her:3

PATIENT HX

Metastatic left breast cancer to bone, liver and LNs
Received Taxol, Herceptin x20 weeks
21/5/2025: Increase in size of one of the right axillary lymph nodes for Sonography-> done on 21/7 -> Few enlarged axillary LNs at level I, the largest one is measuring 18x7mm
CT.CAP 21/7/2025-> RD and multiple lesions, stationary course of the disease

MDT outcome and plan

Consider secondline Xeloda + Tykerb (31/7/2025)

MDT DECISION

CONTINUE FOR 9WKS TAXOL+HERCEPTINX3 CYCLE
NEW CT CAP WITH COMPARSION CT 7\2025

[illegible]	[illegible]		

gnature

GUSTAVE ROUSSY
INTERNATIONAL

Patient's Name:	AFAF AHMED ALSAYED
Date:	2026-03-30
Referred by:	Prof.Dr. CAP+C
Hospital:	**Goustave Roussey International Egypt**

MULTISLICE POST CONTRAST CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:

TECHNIQUE: Multislice axial CT images of the chest, abdomen and pelvis obtained from the lower neck to the symphysis pubis with multiplanar reconstruction. IV contrast was used.

FINDINGS:

In reviewing the previous study report dated 4-10-2025

(I) Chest:

- Still noted bilateral mosaic attenuation of both lungs
- No sizable parenchymal nodules, cavitary lesions, consolidative patches or bronchiectatic changes.
- No CT evidence of hilar or mediastinal lymph node enlargement.
- No pleural or pericardial collections seen.
- No gross cardioemagly
- Still noted left breast UOQ lesion and thickened skin ,

(II) Abdomen and pelvis:

- The liver is enlarged in size showing homogenous CT density with no hepatic focal lesions
- Patent and homogenously enhancing portal vein.
- No intra or extra hepatic biliary radicles dilatation.
- Normal CT appearance of the spleen, pancreas, suprarenal glands, both kidneys ,apart from left renal cortical cyst, aorta and IVC.
- Regular filling of the urinary bladder showing no stones, masses or diverticulae.

GUSTAVE ROUSSY
INTERNATIONAL EGYPT

Patient's Name:	AFAF AHMED ALSAYED
Date:	2026-03-30
Referred by:	Prof.Dr. CAP+C
Hospital:	**Goustave Roussey International Egypt**

- Small fat/omentum containing right inguinal hernia is noted.
- Unremarkable other pelvic organs.
- No retro-crural, para-aortic or pelvic lymphadenopathy.
- No ascites.
- L3 small sclerotic lesion is still noted.
- Dorsolumbar spondylotic changes are noted.

OPINION:

- **Follow up status showing rather stationary course since last study**

Dr.Nourhan Ewis

Dr.Nourhan Ewis

		Request No	: 13003628
		Request Date	: 13/ /202
	Female	Verificatio Date	: 21/ /202
Age	:62 Y	Financial Category	:
Referring Doc :		Department	:Out Patient
		Registered by	:Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Left breast cancer.
Nature of Specimen: Slice revision

Gross Examination:
One paraffin block labled by (P012-25) and one H&E slide were received.

Microscopic Examination:
* Histological type: Invasive duct carcinoma.
* Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+2) Grade II.
* DCIS: Focally positive.
* Angioinvasion: Could not be detected within the limits of biopsy received

Diagnosis:
Left breast mass, Slide revision:
Invasive duct carcinoma, Grade II with focal DCIS.
For further IPT (ER, PR, Her2 and Ki67).

Signature
Prof. Dr. Lubna Omar
Dr. Menna Nabil

Verified by :
Dr.Heba Hasan

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name	: [illegible]	Request No.	: 13003629
Patient No.	: 27206260102459	Request Date	: 13/01/2025
Path No.	: SR-105/25	Verification Date	: 29/01/2025
Sex	: Female	Financial Category	: [illegible]
Age	: 52 Y	Department	: Out Patient
Referreing Doc.	:	IHC No.	:
		Registered by	: Mrs.Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast mass.
Nature of Specimen: Slide revision

Technique:
- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) stained by rabbit monoclonal antibodies using Ultraview HRP DAB as chromogen and hemaloxylin as counter-stain. The sections were treated against: ER clone (1E2), PR clone (SP1), Her2 clone (4B5) and Ki67 clone (30-9).

Results:
* IPT (Positive control showed appropriate reactivity):
- ER: No nuclear reaction in tumor cells (Allred score 0/8).
- PR: No nuclear reaction in tumor cells (Allred score 0/8).
- Her2: Strong complete membrane reaction in>10 % tumor cells (score 3+).
- Ki67: 10% nuclear reaction of tumor cells.

Conclusion:
Left breast cancer, slide revision:
Invasive duct carcinoma, Grade II, with focal DCIS.
IP1:
- ER: Negative (score 0/8).
- PR: Negative (score 0/8).
- Her2: Positive (score 3+).
- Ki67: Positive In 10% of tumor cells.
* Scoring system: "ER, PR"
ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0 no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and

Verified by :
Dr.Toka Medhat

غرفة رقم (4) - الدور الأول - مبنى رقم (387) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 387- Nile Corniche - Misr El Qadema - Cairo, Egypt

Scanned with CamScanner

Patient Name	الأستاذة / هناء احمد السيد عرجوي		
Patient No.	272062601024G9	Request No.	13003629
Path No.	SR-105/25	Request Date	13/01/2025
Sex	Female	Verification Date	29/01/2025
Age	52 Y	Financial Category	تردد دار شكم - بنك خرطة
Referring Doc.		Department	Out Patient

Registered by Mrs.Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10%of the invasive tumor cells or complete and circumferential membrane staining that in intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature
Prof. Dr.Gamal Abdelatty
Dr Heba Hassan

Verified by :
Dr.Toka Medhat

Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema- Cairo, Egypt
غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر

Scanned with CamScanner

Patient name: Ayat Ahmed
Date: 4/7/2025
Prof.Dr. Samar Khamis

US Examination Of Both Breasts Revealed:

- N.R. Mixed fibro-fatty-glandular mesh parenchyma
- An ill-defined hyperdense lesion is seen at the UOQ of the left breast corresponding by US to an ill defined globular shaped hetrogenous hypoechoic lesion is seen at 2 o'clock surrounded by echogenic halo and causing posterior shadowing. It measures about 44 x 32 x 19 mm. associated mild edematous changes manifested by parenchymal coarsening and skin thickening are noted.
- The left axillary region shows multiple enlarged lymph nodes showing increased cortical thickness, and yet preserved fatty hila, the largest measures about 22 x 12 mm.
- No right breast solid or cystic lesions.
- Normal central ducts system.
- Normal right skin thickness and contour as well as nipple areola complexes.
- Right non specific axillary lymph nodes showing preserved fatty hila.

Opinion:

- **Left breast UOQ suspicious lesion as described (BIRADS IV); for histopathological correlation.**

Best regards,
Dr. Esraa Hatem

BIRADS scoring is the breast imaging and reporting data system for classifying the radiological diagnosis of breast lesions:

BIRADS 0: Needs further investigations.
BIRADS 1: Normal findings.
BIRADS 2: Benign.
BIRADS 3: Probably benign.
BIRADS 4: Probably malignant.
BIRADS 5: Malignant.
BIRADS 6: Proven pathologically malignant.

Dr.Esraa Hatem
Radiology specialist

أ.د/ عبله سيد محمود عبد العليم
دكتوراه واستشاري الباثولوجي
المستشفى الجامعي كليه الطب
البشري

أ.د/ إيمان أحمد عبد الوجود
دكتوراه واستشاري الباثولوجي
المستشفى الجامعي كليه الطب
البشري

Patient Name:

Date: Age: 51 y Gender: Female

Referred By Prof. Dr.

Source of the specimen: Tru-Cut Biopsy

Clinical Diagnosis: Left Breast Mass

Code Number: P0612 -25

Date of Visit: 5-1-2025 Report Issued At: 9-1-2025

PATHOLOGY REPORT

Gross Pathology:

- Received are multiple white fibrous cores of average length 1 cm, totally submitted.

Histology:

- Examination of the specimen received revealed breast tissue cores infiltrated by malignant epithelial cells displaying moderate degree of anaplasia arranged in nests, individual cells surrounded by desmoplastic stroma.
- Positive for lymphovascular invasion.
- Mild perineural lymphocytosis.
- No lobular carcinoma components were detected.

Diagnosis:

Left Breast Mass, Tru-Cut Biopsy

Features of Invasive Ductal Carcinoma, Grade III

Immunophenotyping: (ER, PR, HER2 NEU & Ki67).

Eman A. Abd Elmaoged

Abla S. Mahmoud

من الابراهيمية امام مخرج شهاب مصر - بني سويف
ت/ 01017211026

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
الهيئة العامة للمراكز الطبية المتخصصة
مستشفى كورام دار السلام
ادارة الاشعة

Patient Name: Afaf Ahmed Al Sayed Eisawy	Date : 21/05/2025
Exam : CT CAP(CT Chest Abd. Pelvis)	Age : 53 Year(s)
Medical ID: 108/2025	Pat. ID: 96847

NON CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

Findings:

Compared to 13/1/2025

I Chest:-

- **Regressive course** of the:
  1. Left breast retroareolar irregular spiculated mass lesion seen measuring 2 X 1.28 cm compared to 4 X 5 cm seen reaching the nipple and thickened skin.
  2. Pathologically enlarged left axillary level I and II lymph nodes being subcm in current study.
- **Increase in size** of one of the right axillary level I lymph nodes reaching 9 X 12 mm.
- Non appreciated forenoted enlarged left lower deep cervical lymph nodes in current exam field.
- No sizable pulmonary nodules (apart from currently noted left basal 3 mm nodule), masses, areas of consolidation or cavitary lesions.
- **Regressive course** of the enlarged mediastinal lymph nodes being subcm.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II Abdomen & pelvis:

- The liver is of average size with regressive course of the bilobar hypodense hepatic focal lesions being hardly appreciated in current non contrast study.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

***Scanned skeleton:** **more sclerosis** L5 vertebral body tiny lytic lesion suggesting favorable response.
Stable L3 sclerotic lesion mostly bone island.

Opinion:

- **Increase in size of one of the right axillary lymph nodes for sonographic assessment.**
- **Overall regressive course as described.**

Dr. Mohamed Mohsen, M.D.

Scanned with CamScanner

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

09/22/2026

MDT FORM

Name:	Afaf Ahmed El-Sayed .	**MRN:**	648
Age:	54 Y	**ID:**	27206260102469
Menopausal Sta		**Phone:***	01062013501
NRS SCORE:		**Marital Status:**	
BMI:		**Family History:**	
Gender			
CO Morbidity:		**Reason Of MDT:**	
		Progressive disease	
Surgical History:*		**ECOG Performance Status**	
		○ 0 ○ 1 ◉ 2 ○ 3 ○ 4	

Clinical Data:

BX: IDC

ER: 0/8 **PR:** 0/8 **HER2:** 3+ **KI67:** 10%

R L

TNM

History:
MBC to bone and lymph nodes
Received Paclitaxel - Herceptin
Then Tykerb - Xeloda

Mammography:

Right Side				Left Side			
Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4

Biomarker/Molecular

Mets Workup:
Complaining from headache and slurred speech
O/E oral moniliasis
Brain MRI without contrast: Multiple SOL
CT CAP 19/8: SD of L# sclerotic lesion
Bone Scan 20/8: No mets seen
To start Diflucan, Methablogen
New Brain MRI + C
WCI

Clinical Staging:

Pathological Staging:

Recommendations:

MDT Outcome & Plan
New Brain MRI + C
WCI
Refer for TDM-1 (NGO)
For Gemcitabine - Carboplatin meanwhile

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY

جوستاف روسي

Patient Name	: عفاف احمد السيد	**Patient Id**	: 3474
Patient Info.	: Female 53Y	**VisitId**	: 5213
Department	: Out Clinic	**Collection Date**	: 04/12/2025 11:39:29
Profile NO.	: 108-2025	**Reporting Date**	: 04/12/2025 11:44:10

COMPLETE BLOOD PICTURE

Test		Result	Unit	Reference Range
CBC				
Hemoglobin (HB)	:	13.2	g/dL	12 - 16
RBCs	:	4.61	10^6/cmm	3.8 - 4.8
Haematocrit (HCT)	:	39.6	%	36 - 46
MCV	:	85.9	fl	76 - 95
MCH	:	28.6	pg	26 - 32
MCHC	:	33.3	g/dL	32 - 36
RDW	:	H 15.80	%	11.6 - 14.5
Platelet count	:	216	10^3/cmm	150 - 450
WBCs	:	L 3.44	10^3/cmm	4 - 10

Differential white cell Count	*Relative*		*Absolute*	
	Relative (%)	*R.Range*	*Absolute (x 10^3/ul)*	*R.Range*
Neutrophils	L 39.50	40 - 70	L 1.36	2 - 7
Lymphocytes	42.50	20 - 45	1.46	1 - 3
Monocytes	6.10	2 - 8	0.21	Up To 1.o
Eosinophils	H 11.70	1 - 6	0.40	Up To 500
Basophils	0.20	0 - 1	0.01	Up To 100

Signature
Dr. Sahar Mansour

جوستاف روسي

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

البنك المصرية من أجل صحة مصر

MDT FORM / /20 G

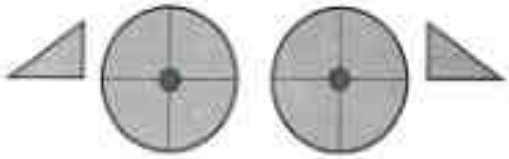
Name	[illegible]	MRN	108/2025
		ID	
Age	52	Phone	
Menopausal status		Marital status	
NRS SCORE		Family history	
BMI		REASON OF MDT	
CO morbidity			

Clinical Data

BX: IDC GII

ER:	PR:	HER 2:	Ki67:
0/8	0/8	+3	

R L



Sonomammography:

Mets Workup: Mets breast Cancer to Liver & LN TNM

received 1st line taxol weekly + Herceptin (Cycle 1 to be 1-e. AP Cycle 1 5-2025)

last CTCAP → SD.

Recommendations: for MDT.

31/7/2025. Cont. the 2nd line Taxol-Tykerb

MDT OUTCOME & PLAN

SIGNATURE					
MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

GUSTAVE ROUSSY

7-8-2025

elevate private equity

MDT

Patient Name	طالب احمد السيد	Age	52
Sex	Female	Special habits	
Family History		Co-morbidity	
Surgical History		Telephone	01281289063
Menopausal Status	POSTPREMENOPAUSAL	Marital Status	
MEDICAL ID	108/2025	ID	27206260102469

Pathology Bx: IDC GII
ER:0
PR:0
Her:3

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MDT outcome and plan

Consider secondline Xeloda + Tykerb (31/7/2025)

MDT DECISION

CONTINUE FOR 9WKS TAXOL+HERCEPTINX3 CYCLE
NEW CT CAP WITH COMPARSION CT 7\2025

MDT	د/ايمن مصطفى		

gnature

جوستاف روسي

Patient Name	: عفاف احمد السيد	**Patient Id**	: 3474
Patient Info.	: Female 53Y	**VisitId**	: 5213
Department	: Out Clinic	**Collection Date**	: 04/12/2025 11:39:29
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	Relative (%)	*R.Range*	*Absolute (x 10^3/ul)*	*R.Range*
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Lymphocytes	42.50	20 - 45	1.46	1 - 3
Monocytes	6.10	2 - 8	0.21	Up To 1.o
Eosinophils	H 11.70	1 - 6	0.40	Up To 500
Basophils	0.20	0 - 1	0.01	Up To 100

Signature
Dr. Sahar Mansour

جوستاف روسي

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

البنك المصرية من أجل صحة مصر

MDT FORM / /20 G

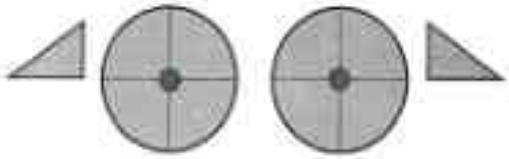
Name	[illegible]	MRN	108/2025
		ID	
Age	52	Phone	
Menopausal status		Marital status	
NRS SCORE		Family history	
BMI		REASON OF MDT	
CO morbidity			

Clinical Data

BX: IDC GII

ER:	PR:	HER 2:	Ki67:
0/8	0/8	+3	

R L



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Mets Workup: Mets breast Cancer to Liver & LN TNM

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Recommendations: for MDT.

31/7/2025. Cont. the 2nd line Taxol-Tykerb

MDT OUTCOME & PLAN

SIGNATURE					
MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

Patient Name	: الأستاذه / عفاف احمد السيد عيسوي	Request No.	: 13003629
Patient No.	: 27206260102469	Request Date	: 13/01/2025
Path No.	: SR-105/25	Verification Date	: 21/01/2025
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 52 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Left breast cancer.
Nature of Specimen: Slide revision.

Gross Examination:
One paraffin block labled by (P012-25) and one H&E slide were received.

Microscopic Examination:
* Histological type: Invasive duct carcinoma.
* Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+2) Grade II.
* DCIS: Focally positive.
* Angioinvasion: Could not be detected within the limits of biopsy received

Diagnosis:
Left breast mass, Slide revision:
Invasive duct carcinoma, Grade II with focal DCIS.
- For further IPT (ER, PR, Her2 and Ki67).

Signature
Prof. Dr. Lubna Omar
Dr. Menna Nabil

ERM036748357

Verified by :
Dr.Heba Hasan

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

GUSTAVE ROUSSY

7-8-2025

elevate private equity

MDT

Patient Name	طالب احمد السيد	Age	52
Sex	Female	Special habits	
Family History		Co-morbidity	
Surgical History		Telephone	01281289063
Menopausal Status	POSTPREMENOPAUSAL	Marital Status	
MEDICAL ID	108/2025	ID	27206260102469

Pathology Bx: IDC GII
ER:0
PR:0
Her:3

PATIENT HX: Metastatic left breast cancer to bone, liver and LNs
Received Taxol, Herceptin x20 weeks
21/5/2025: Increase in size of one of the right axillary lymph nodes for Sonography-> done on 21/7 -> Few enlarged axillary LNs at level I, the largest one is measuring 18x7mm
CT.CAP 21/7/2025-> RD and multiple leslons, stationary course of the disease

MDT outcome and plan: Consider secondline Xeloda + Tykerb (31/7/2025)

MDT DECISION: CONTINUE FOR 9WKS TAXOL+HERCEPTINX3 CYCLE
NEW CT CAP WITH COMPARSION CT 7\2025

MDT	د/ايمن مصطفى		

gnature

Patient Name	: الأستاذه / عفاف احمد السيد عيسوي	Request No.	: 13003629
Patient No.	: 27206260102469	Request Date	: 13/01/2025
Path No.	: SR-105/25	Verification Date	: 21/01/2025
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 52 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Left breast cancer.
Nature of Specimen: Slide revision.

Gross Examination:
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Invasive duct carcinoma, Grade II with focal DCIS.
- For further IPT (ER, PR, Her2 and Ki67).

Signature
Prof. Dr. Lubna Omar
Dr. Menna Nabil

ERM036748357

Verified by :
Dr.Heba Hasan

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
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IEYA SCAN

مركز
هي للأشعة

NAME : Afaf Ahmad
DATE: 8/1/2025
PROF .DR; SaedAlbehery

Bilateral MAMMOGRAM and HIGH RESOLUTION ULTRASOUND EXAMINATION

- **ACR B breast parenchyma**
- **An ill-defined spiculated radio-dense lesion is seen at left breast at upper outer quadrant ,associated diffuse increased skin thickness is seen**
- **Complementary u/S reveals: An ill-defined hypoechoic lesion is seen at the upper outer quadrant of the left breast at 12-3 o'clock measuring about 35x25mm ,few internal vascularity is noted by color Doppler study .The lesion is seen surrounded by echogenic breast parenchyma (desmoplastic reaction).multiple tiny echogenic foci seen inside likely microcalcification**
- **No dilated lactiferous ducts**
- Norma right skin thickness and contour.
- Normal nipple areola complex.
- **Multiple enlarged left axillary lymph nodes, the largest measures about 29x25mm ,some show effaced fatty hila**

OPINION:

- ***Left breast upper outer quadrant soft tissue lesions likely malignant associated with pathological left axillary lymph nodes for histopathological correlation (BIRADS 5)***

Much obliged,
DR .Eman AbdelRahman ,MD

01119598889 - 01005425312 0822167074
بني سويف - ش احمد عرابي برج زيادة 2 - أمام قصر الثقافة
بني سويف - أمام أجيال للملابس

Patient Name: AFAF AHMED ALSAYED

Patient ID: 32**154

Gems
Nuclear Imaging Environment

Study Name: BONE SCAN

Date & Time: 2/2/2015

Manufacturer Model: VARICAM





فيجا لاب

لتحاليل الخلايا والأنسجة والاورام

ا.د/ عبله سيد محمود عبد العليم
دكتوراه واستشاري الباثولوجي
المستشفى الجامعي كليه الطب البشري

ا.د/ إيمان أحمد عبد الموجود
دكتوراه واستشاري الباثولوجي
المستشفى الجامعي كليه الطب البشري

Patient's Data

Patient Name: عفاف أحمد السيد

Age: 51 y Gender: Female

Referred By Prof. Dr. أ.د سعيد البحيري

Nature of the specimen: Tru-Cut Biopsy

Clinical Diagnosis: Left Breast Mass

Code Number: P0012 -25

Received At : 5- 1 - 2025

Report issued At : 9-1-2025

PATHOLOGY REPORT

Gross Pathology:

Multiple grayish white tissue cores of average length 1 cm, totally submitted.

Histology:

- Examination of the specimen received revealed breast tissue cores infiltrated by malignant epithelial cells displaying moderate degree of anaplasia, arranged in nests, individual cells surrounded by desmoplastic stroma.
- Positive for lymphovascular invasion.
- Mild peritumoral lymphocytosis
- No Intraductal components were detected.

Diagnosis:

Left Breast Mass; Tru-Cut Biopsy

Features of Invasive Duct Carcinoma, Grade III

For Immunophenotyping (ER, PR, HER/2 NEU & KI67).

معمل فيجا لاب للتحاليل الخلايا والأنسجة والأورام

Eman A. Abd Elmaogod

Abla S. Mahmoud

Patient name: Ms. Afaf Ahmad Al Sayed

Referred by: Prof. Dr.

Date: Feb ,03.2021

Tc^{99m}-MDP- Bone scan

Procedure:

- Anterior and posterior whole body scans were acquired 10 minutes and 2 hours post injection of 20 mCi of the tracer.

Findings:

- No detected increased abnormal tracer uptake over the scanned skeleton, to account for serious bone lesions.
- Osteoarthritic changes of shoulder and right knee joints.
- Scoliotic changes of the dorsal spine.

Opinion

- **No scintigraphic evidence of bone deposits throughout the scanned skeleton.**

Mahmoud Shaban Abd Al-Alim, MD, EBNM

European Board of Nuclear Medicine

فيبا لاب

لتحاليل الخلايا والأنسجة والاورام

ا.د/ عبله سيد محمود عبد العليم
دكتوراه واستشاري الباثولوجي
المستشفى الجامعي كليه الطب البشري

ا.د/ إيمان أحمد عبد الموجود
دكتوراه واستشاري الباثولوجي
المستشفى الجامعي كليه الطب البشري

Patient's Data

Patient Name: عفاف احمد السيد

Age: 51 y Gender: Female

Referred By Prof. Dr. أ.د سعيد البحيري

Nature of the specimen: Tru-Cut Biopsy

Clinical Diagnosis: Left Breast Mass

Code Number: P0012 -25

Received At : 5- 1 - 2025

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- Positive for lymphovascular invasion.
- Mild peritumoral lymphocytosis
- No Intraductal components were detected.

Diagnosis:

Left Breast Mass: Tru-Cut Biopsy

Features of Invasive Duct Carcinoma, Grade III

For Immunophenotyping (ER, PR, HER/2 NEU & KI67).

Eman A. Abd Elmaogod

Abla S. Mahmoud

ش الابراهيمية امام معرض شباب مصر – بني سويف

ت/ 01017211026

Patient name: Afaf Ahmed
Date: 4/1/2025
Prof.Dr. Samar Khamis

US Examination Of Both Breasts Revealed:

- ACR 2: Mixed fibro-fatty-glandular breast parenchyma.
- **An ill-defined hyperdense lesion is seen at the UOQ of the left breast corresponding by US to an ill defined globular shaped hetrogenous hypoechoic lesion is seen at 2 o'clock surrounded by echogenic halo and causing posterior shadowing. It measures about 44 x 32 x 19 mm. associated mid edematous changes manifested by parenchymal coarsening and skin thickening are noted.**
- **The left axillary region shows multiple enlarged lymph nodes showing increased cortical thickness, and yet preserved fatty hila, the largest measures about 22 x 12 mm.**
- No right breast solid or cystic lesions.
- Normal central ducts system.
- Normal right skin thickness and contour as well as nipple areola complexes.
- Right non specific axillary lymph nodes showing preserved fatty hila.

Opinion:

- **Left breast UOQ suspicious lesion as described (BIRADS IV); for histopathological correlation.**

Best regards,
Dr. Esraa Hatem

***BIRADS** scoring is the **breast** imaging and reporting data system for classifying the radiological diagnosis of **breast** lesions:*

BIRADS 0: Needs further investigations.
BIRADS 1: Normal findings.
BIRADS 2: Benign.
BIRADS 3: Probably benign.
BIRADS 4: Probably malignant.
BIRADS 5: Malignant.
BIRADS 6: Proven pathologically malignant

Dr.Esraa Hatem
Radiology specialist

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
إدارة الأشعة

Patient Name: Afaf Ahmed Al Sayed Elsawy	Date : 13/01/2025
Exam : C.T ABDOMEN &PELVIS	Age : 52 Year(s)
Medical ID: 108/2025	Pat. ID: 96847

NON CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

Findings:

I Chest:-

- Left breast retroareolar irregular spiculated mass lesion seen measuring 4 X 5 cm seen reaching the nipple and thickened skin.
- Pathologically enlarged left axillary level I and II lymph nodes the largest is seen measuring 1.3 X 1.9 cm.
- Enlarged left lower deep cervical lymph nodes reaching 1.5 cm.
- No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- Enlarged mediastinal lymph nodes the largest is prevascular measuring 25 X 12 mm..
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II Abdomen & pelvis:

- The liver is of average size with bilobar hypodense hepatic focal lesions the largest is at left lobe segment II measuring 1.2 X 1.7 cm.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

***Scanned skeleton:** L5 vertebral body tiny lytic lesion.

Opinion:

- **Locally advanced left breast malignant process with pathological ipsilateral axillary lymphadenopathy.**
- **Mediastinal and left lower deep cervical lymph nodes worrisome of metastatic deposits.**

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Specialized Medical Centers
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Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Afaf Ahmed Al Sayed Esawy	Date : 13/01/2025
Exam : C.T ABDOMEN &PELVIS	Age : 52 Year(s)
Medical ID: 108/2025	Pat. ID: 96847

- **Bilobar hepatic focal lesions for triphasic CT assessment.**
- **L5 tiny lytic lesion.**
- **For correlation with the unavailable previous studies.**

Dr. Mohamed Mohsen, M.D.

Patient Name	الأستاذ / عبدالله احمد السيد عيسوي	Request No.	13003629
Patient No.	272082901024 69	Request Date	13/01/2025
Path No.	SR-105/25	Verification Date	29/01/2025
Sex	Female	Financial Category	اورام دار السلام - دفعة الدولة
Age	52 Y	Department	Out Patient
Referring Doc.		Registered by	Mrs.Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/mocerate and within < 10%of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature
Prof. Dr.Gamal Abdelatty
Dr Heba Hassan

Verified by :
Dr.Toka Medhat

غرفة رقم (4) – الدور الأول – مبنى رقم (307) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 307- Nile Corniche - Misr El Qadema - Cairo, Egypt

Scanned with CamScanner

Patient Name	: الأستاذه / هطف احمد السيد عيسوي	Request No.	: 13003629
Patient No.	: 27206260102469	Request Date	: 13/01/2025
Path No.	: SR-105/25	Verification Date	: 29/01/2025
Sex	: Female	Financial Category	: اورام دار السلام - نقابة الشرطة
Age	: 52 Y	Department	: Out Patient
Referreing Doc.	:	IHC No.	:
		Registered by	: Mrs.Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast mass.
Nature of Specimen: Slide revision

Technique:
- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) stained by rabbit monoclonal antibodies using Ultraview HRP DAB as chromogen and hematoxylin as counter-stain. The sections were treated against: ER clone (1E2), PR clone (5P1), Her2 clone (4B5) and Ki67 clone (30-9).

Results:
* IPT (Positive control showed appropriate reactivity):
- ER: No nuclear reaction in tumor cells (Allred score 0/8).
- PR: No nuclear reaction in tumor cells (Allred score 0/8).
- Her2: Strong complete membrane reaction in>10 % tumor cells (score 3+).
- Ki67: 10% nuclear reaction of tumor cells.

Conclusion:
Left breast cancer, slide revision:
Invasive duct carcinoma, Grade II, with focal DCIS.
IPT:
- ER: Negative (score 0/8).
- PR: Negative (score 0/8).
- Her2: Positive (score 3+).
- Ki67: Positive in 10% of tumor cells.

* Scoring system: "ER, PR"
ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0 no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and

Verified by :
Dr.Toka Medhat

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Scanned with CamScanner

Patient Name	:الأستاذه / عفاف احمد السيد حجوري	Request No.	:13003629
Patient No.	:27206260102469	Request Date	:13/01/2025
Path No.	:SR-105/25	Verification Date	:21/01/2025
Sex	:Female	Financial Category	:فورام دار السلام - نقابة المرئة
Age	:52 Y	Department	:Out Patient
Refereing Doc :		Registered by	:Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Left breast cancer.
Nature of Specimen: Slide revision.

Gross Examination:
One paraffin block labled by (P012-25) and one H&E slide were received.

Microscopic Examination:
* Histological type: Invasive duct carcinoma.
* Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+2) Grade II.
* DCIS: Focally positive.
* AngioInvasion: Could not be detected within the limits of biopsy received

Diagnosis:
Left breast mass, Slide revision:
Invasive duct carcinoma, Grade II with focal DCIS.
- For further IPT (ER, PR, Her2 and Ki67).

Signature
Prof. Dr. Lubna Omer
Dr. Menna Nabil

Verified by :
Dr.Heba Hasan

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Scanned with CamScanner

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

السيدات المصرية هي دايما مصر

MDT FORM 1/2/2025 GM

Name	عناق احمد السيد عيسوى	MRN	٢٠٢٥١٠٨
Age	52	ID	٢٧٠٢٠٠٤٠٢٢٦
Menopausal status		Phone	٠١٢٨١٤٨٤٠٦٢
NRS SCORE		Marital status	
BMI		Family history	
CO morbidity		REASON OF MDT	NEW

Clinical Data

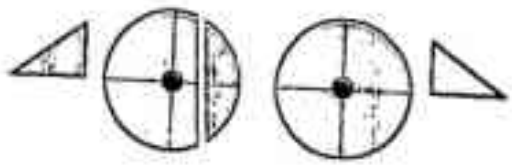
BX: IDC

ER:	PR:	HER 2:	KI67:
0	0	+3	

Sonomammography:

Mets Workup:

R L



TNM

- Mediastinal L.N
- Bilobar F.L

Recommendations:

MDT OUTCOME & PLAN

1/2/2025 → for - 1st line Taxol (herceptin IV (3C)
For Re evaluate.
- baseline CT chest, PA etcontra

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

جوستافروسي

مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

البنت المصرية هي صحة مصر

MDT FORM / /٢٠ G

Name	كفاني محمد السيد عويس	MRN	
		ID	
Age	52	Phone	
Menopausal status		Marital status	
NRS SCORE		Family history	
BMI		REASON OF MDT	
CO morbidity			

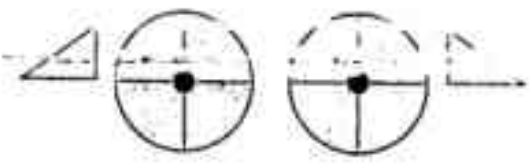
Clinical Data

BX: IDC CD

ER:	PR:	HER 2:	KI 67:
0	0	+3	

Sonomammography:

R L



Mets Workup: Metastatic Rt breast Ca to bone, liver, L.N. Received Taxol / Herceptin x20 week CT.CAP 21/5/20 → RD of multiple lesion

TNM

Recommendations: RD of axillary L.N. For MDT for next plan of Management

21/7/20

MDT OUTCOME & PLAN

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

GUSTAVE ROUSSY
INTERNATIONAL

Patient Name: Afaf Ahmed Al Sayed Elsawy	Date : 21/07/2025
Exam : Axillary u/s	Age : 53 Year(s)
Medical ID: 108/2025	Pat. ID: 96847

Soft tissue ultrasound examination of right axilla

- *Few enlarged axillary LNs at level I, the largest one is measuring 18x7mm showing relatively thickened cortex 3.5mm? reactionary for follow up*
- *Normal skin thickness and subcutaneous tissues*
- *Normal vascular structure*

- Signature Dr . Nermien Ramadan
- 21/07/2025 12:35 PM

Patient Name: Afaf Ahmed Al Sayed Eisawy	Date : 07/08/2025
Exam : Item 1	Age : 53 Year(s)
Medical ID: DAR ELSALAM HOSPITAL^BRAIN	Pat. ID: 96847

MRI EXAMINATION OF THE BRAIN

MR Technique:

- Axial T1, T2 and FLAIR WI's
- Sagittal T1 WI
- Coronal T2 WI
- Axial DWI and ADC
- Post contrast T1 series

MR findings:

- W dened cortical sulci, basal cisterns, cerebellar folia and extra-axial CSF spaces.
- Mild bilateral symmetrical dilatation of the supra-tentorial ventricular system
- Nc cerebral, cerebellar or brain stem areas of altered MR signal.
- Nc midline shift or deformity.
- No extra axial collections or masses.
- No cerebello pontine angle masses.
- No gross posterior fossa abnormalities.

Opinion:

- **Mild age matched brain involutional changes.**

Dr.Zeyad El-Sayed,FEBR

GUSTAVE ROUSSY INTERNATIONAL EGYPT

Patient Name: Afaf Ahmed Al Sayed Eisawy

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

Findings:

Compared to 21/5/2025

I Chest:-

- **Stationary course** of the:
  1. Left breast retroareolar irregular spiculated mass lesion seen measuring 2 X 1.28 cm compared to 4 X 5 cm seen reaching the nipple and thickened skin.
  2. Pathologically enlarged left axillary level I and II lymph nodes being subcm in current study.
- **Stationary course** of one of the right axillary level I lymph nodes reaching 9 X 12 mm.
- Non appreciated forenoted enlarged left lower deep cervical lymph nodes in current exam field.
- No sizable pulmonary nodules (apart from currently noted left basal 3 mm nodule), masses, areas of consolidation or cavitary lesions.
- **Stationary course** of the enlarged mediastinal lymph nodes being subcm.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.



II Abdomen & pelvis:

- The liver is of average size with regressive course of the bilobar hypodense hepatic focal lesions being hardly appreciated in current non contrast study.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

***Scanned skeleton:** more sclerosis L5 vertebral body tiny lytic lesion suggesting favorable response.

Stable L3 sclerotic lesion mostly bone island.

Opinion:

- **Stationary course since the last study.**

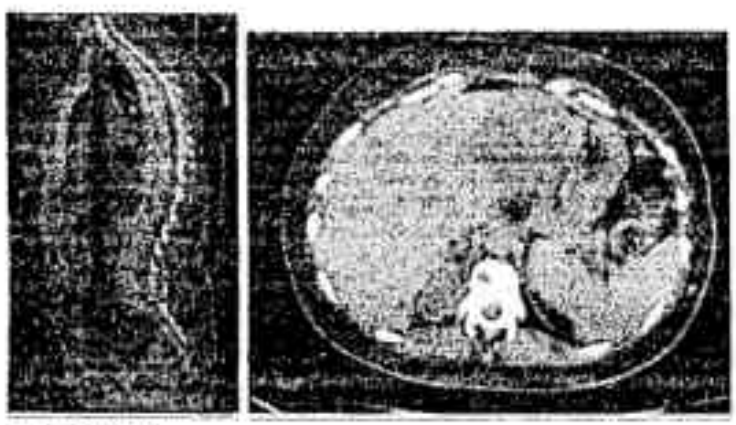
Date 12

Dr. AYA GABR

Name : Afaf El Sayed
Date : 20-7-2025

MULTI-SLICE NON CONTRAST CT SCAN OF THE ABDOMEN AND PELVIS: - PRIMARY REPORT

- ***Known case of right breast cancer. Compared to the previous study dated 5-3-2025, the current study revealed:***



Findings:

- The liver is still seen mildly enlarged (16 cm) with homogenous CT texturestill showing hypodense rather cystic focal lesion occupying segment II and III currently measures about 8x7.8x8.5 cm (compared to 8.2x7.5x7.5 cm), showing thin septations.
- No intra-hepatic or extra-hepatic biliary radicle dilatation.
- Normal size (11.5 cm) and parenchymal texture of the spleen with no sizable focal lesions seen. Still noted two splenules.
- Normal size, parenchymal thickness and density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas and both supra renal glands.
- Still noted multiple subcentemetric abdominal lymph nodes (para-aortic, mesenteric, right common iliac, left external iliac and bilateral inguinal).
- No ascites.
- Average filling of the urinary bladder with no definite stones or masses.
- No gross adnexal lesions.
- Still noted multiple axial and appendicular skeleton lytic lesions with no associated soft tissue component or structural vertebral collapse.

Ministry of Health
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Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Afaf Ahmed Al Sayed Eisawy	Date : 21/05/2025
Exam : CT CAP(CT Chest Abd. Pelvis)	Age : 53 Year(s)
Medical ID: 108/2025	Pat. ID: 96847

NON CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

Findings:

Compared to 13/1/2025

I Chest:-

- **Regressive course** of the:
  1. Left breast retroareolar irregular spiculated mass lesion seen measuring 2 X 1.28 cm compared to 4 X 5 cm seen reaching the nipple and thickened skin.
  2. Pathologically enlarged left axillary level I and II lymph nodes being subcm in current study.
- **Increase in size** of one of the right axillary level I lymph nodes reaching 9 X 12 mm.
- Non appreciated forenoted enlarged left lower deep cervical lymph nodes in current exam field.
- No sizable pulmonary nodules (apart from currently noted left basal 3 mm nodule), masses, areas of consolidation or cavitary lesions.
- **Regressive course** of the enlarged mediastinal lymph nodes being subcm.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II Abdomen & pelvis:

- The liver is of average size with regressive course of the bilobar hypodense hepatic focal lesions being hardly appreciated in current non contrast study.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

***Scanned skeleton:** **more sclerosis** L5 vertebral body tiny lytic lesion suggesting favorable response.
Stable L3 sclerotic lesion mostly bone island.

Opinion:

- **Increase in size of one of the right axillary lymph nodes for sonographic assessment.**
- **Overall regressive course as described.**

Dr. Mohamed Mohsen, M.D.

إسم المريض ثلاثي: .....
رقم الملف: .....

جوستاف روسي

Chronic pain clinic referral form

Mark the inclusion criteria : *at least one*

Persistent Pain Management Service (PPMS) input with patient **approval** to follow up for at least ٦ months

Patient is cognitively capable, aware and agreeable to participate in the Pain management Program including self-management strategies and interdisciplinary teammembers as an option.

- **Worsening** post-surgical pain of more than ٣ **months** duration (where a post-operative complication has been excluded and conventional analgesics max dose given)
- Newly diagnosed or suspected complex regional pain syndrome (CRPS) greater than ٦**weeks** after the triggering event
- Neuropathic pain (uncontrolled)
- Escalation of **symptoms** associated with escalation of **functional disability** (school attendance, engagement in physical activity, sleep, mood changes, **frequent** emergencydepartment attendance)

Note the exclusion criteria :

- Ongoing untreated infection source
- Untreated addiction
- Unstable condition in need of inpatient care and monitoring
- Poorly controlled psychotherapy

Referral notes :
52 Yrs old female Patient with Mets breast Cancer
To liver & L.N. on Cht & target (Herceptin) with
R.C. but with Incidental Pain
Please manage based on Acute & Assess regularly
Pain therapy.

Referring Doctor's Name: ..... **Signature:** .....

Date: 3/7/2020 **Time:** 1 a.m.

Please Consider
→ Anti-inflam dose
for ten days

6.10/8/19

→ Neuropathic pain Killer
Gaptin 300mg two days

Patient Name: Afaf Ahmed Al Sayed Eisawy	Date : 21/07/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 53 Year(s)
Medical ID: 108/2025 1720 1000	Pat. ID: 96847

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST PRIMARY REPORT

- ***Known case of right breast cancer. Compared to the previous study dated 5-3-2025. the current study revealed:***



Findings:

- Right breast operative bed still shows an ill-defined soft tissue density resting over the chest wall muscles measuring 3x1.5 cm for clinical and SM correlation.
- Still noted few subcentimetric left axillary lymph nodes.
- Still noted ubcentimetric left supra-clavicular lymph node.
- Still noted right lower lung lobe posterior segment tiny partially calcified pulmonary nodule about 4 mm.
- Still noted right upper and middle lung lobes subpleural reticulations and faint ground glass opacities, likely post radiation sequel.
- Left basal atelectatic bands are still noted.
- No sizable hilar or mediastinal lymph nodes regarding this non contrast study.
- Patent tracheo- bronchial tree.
- Average size of the heart.
- No evidence of pleural or pericardial effusion.

Patient Name: Afaf Ahmed Al Sayed Elsawy	Date : 21/07/2025
Exam : CT CAP+C (CT Chest Abc. Pelvis)	Age : 53 Year(s)
Medical ID: 108/2025 1720 1000	Pat. ID: 96847

OPINION:

- ***Stationary course of the disease process.***

Marina Essam, MSc

مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

MDT FORM 1/2/2025 GM

Name	عفاف احمد السيد عيسوى	MRN	C٤٥١٠٨
Age	52	ID	٢٧٠٤٠٠٠٤٢
Menopausal status		Phone	٠١٤٥١٤٥٤٦٢
NRS SCORE		Marital status	
BMI	[redacted]	Family history	
CO morbidity		REASON OF MDT	NEW

Clinical Data

BX: IDC

ER:	PR:	HER 2:	KI67:
C	0	+3	

Sonomammography:

Mets Workup:

R L

TNM
- Mediastinal L.N
- B.lobar F.L

Recommendations:

MDT OUTCOME & PLAN

1/2/2025 → Fit - 1st line Taxol (herceptin IV) (SC)
Then Letrozole.
- baseline CT chest, FA etcetera

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

جوستافروسي

مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

البنك المصرية هي صحة مصر

MDT FORM / /20 G

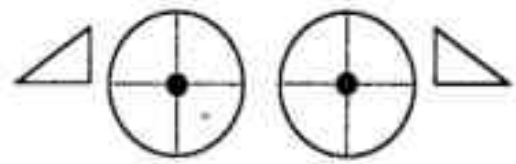
Name	دعاء أحمد السيد	MRN	10812025
		ID	
Age	52	Phone	
Menopausal status		Marital status	
NRS SCORE		Family history	
BMI		REASON OF MDT	
CO morbidity			

Clinical Data

BX: IDC GIII

ER:	PR:	HER 2:	KI67:
0/8	0/8	+3	

R L



Sonomammography:

Mets Workup: Mets breast Cancer to Liver & LN TNM
received 1st line taxol weekly + Herceptin
last CTCAP → SD. (Cycle 4 to be)

Recommendations: for MDT. i.e. BS Cycle 4
5-2025
Dr 31/7/2025. Consider 2nd line Xeloda-Tykerb

MDT OUTCOME & PLAN

SIGNATURE					
MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

Patient Name	: الأستاذه / عفاف احمد السيد عيسوي	Request No.	: 13003629
Patient No.	: 27206260102469	Request Date	: 13/01/2025
Path No.	: SR-105/25	Verification Date	: 21/01/2025
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 52 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Left breast cancer.
Nature of Specimen: Slide revision.

Gross Examination:
One paraffin block labled by (P012-25) and one H&E slide were received.

Microscopic Examination:
* Histological type: Invasive duct carcinoma.
* Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+2) Grade II.
* DCIS: Focally positive.
* Angioinvasion: Could not be detected within the limits of biopsy received

Diagnosis:
Left breast mass, Slide revision:
Invasive duct carcinoma, Grade II with focal DCIS.
- For further IPT (ER, PR, Her2 and Ki67).

Signature
Prof. Dr. Lubna Omar
Dr. Menna Nabil

ERM036748357

Verified by :
Dr.Heba Hasan

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

IEYA SCAN

مركز
هي للأشعة

NAME : Afaf Ahmad
DATE: 8/1/2025
PROF .DR; SaedAlbehery

Bilateral MAMMOGRAM and HIGH RESOLUTION ULTRASOUND EXAMINATION

- **ACR B breast parenchyma**
- **An ill-defined spiculated radio-dense lesion is seen at left breast at upper outer quadrant ,associated diffuse increased skin thickness is seen**
- **Complementary u/S reveals: An ill-defined hypoechoic lesion is seen at the upper outer quadrant of the left breast at 12-3 o'clock measuring about 35x25mm ,few internal vascularity is noted by color Doppler study .The lesion is seen surrounded by echogenic breast parenchyma (desmoplastic reaction).multiple tiny echogenic foci seen inside likely microcalcification**
- **No dilated lactiferous ducts**
- Norma right skin thickness and contour.
- Normal nipple areola complex.
- **Multiple enlarged left axillary lymph nodes, the largest measures about 29x25mm ,some show effaced fatty hila**

OPINION:

- ***Left breast upper outer quadrant soft tissue lesions likely malignant associated with pathological left axillary lymph nodes for histopathological correlation (BIRADS 5)***

Much obliged,
DR .Eman AbdelRahman ,MD

01119598889 - 01005425312 0822167074
بني سويف - ش احمد عرابي برج زيادة 2 - أمام قصر الثقافة
بني سويف - أمام أجيال للملابس

Patient Name: AFAF AHMED ALSAYED

Patient ID: 32**154

Gems
Nuclear Imaging Environment

Study Name: BONE SCAN

Date & Time: 2/2/2015

Manufacturer Model: VARICAM





فيجا لاب

لتحاليل الخلايا والأنسجة والاورام

ا.د/ عبله سيد محمود عبد العليم
دكتوراه واستشاري الباثولوجي
المستشفى الجامعي كليه الطب البشري

ا.د/ إيمان أحمد عبد الموجود
دكتوراه واستشاري الباثولوجي
المستشفى الجامعي كليه الطب البشري

Patient's Data

Patient Name: عفاف أحمد السيد

Age: 51 y Gender: Female

Referred By Prof. Dr. أ.د سعيد البحيري

Nature of the specimen: Tru-Cut Biopsy

Clinical Diagnosis: Left Breast Mass

Code Number: P0012 -25

Received At : 5- 1 - 2025

Report issued At : 9-1-2025

PATHOLOGY REPORT

Gross Pathology:

Multiple grayish white tissue cores of average length 1 cm, totally submitted.

Histology:

- Examination of the specimen received revealed breast tissue cores infiltrated by malignant epithelial cells displaying moderate degree of anaplasia, arranged in nests, individual cells surrounded by desmoplastic stroma.
- Positive for lymphovascular invasion.
- Mild peritumoral lymphocytosis
- No Intraductal components were detected.

Diagnosis:

Left Breast Mass; Tru-Cut Biopsy

Features of Invasive Duct Carcinoma, Grade III

For Immunophenotyping (ER, PR, HER/2 NEU & KI67).

معمل فيجا لاب للتحاليل الخلايا والأنسجة والأورام

Eman A. Abd Elmaogod

Abla S. Mahmoud

Patient name: Ms. Afaf Ahmad Al Sayed

Referred by: Prof. Dr.

Date: Feb ,03.2021

Tc^{99m}-MDP- Bone scan

Procedure:

- Anterior and posterior whole body scans were acquired 10 minutes and 2 hours post injection of 20 mCi of the tracer.

Findings:

- No detected increased abnormal tracer uptake over the scanned skeleton, to account for serious bone lesions.
- Osteoarthritic changes of shoulder and right knee joints.
- Scoliotic changes of the dorsal spine.

Opinion

- **No scintigraphic evidence of bone deposits throughout the scanned skeleton.**

Mahmoud Shaban Abd Al-Alim, MD, EBNM

European Board of Nuclear Medicine

فيبا لاب

لتحاليل الخلايا والأنسجة والاورام

ا.د/ عبله سيد محمود عبد العليم
دكتوراه واستشاري الباثولوجي
المستشفى الجامعي كليه الطب البشري

ا.د/ إيمان أحمد عبد الموجود
دكتوراه واستشاري الباثولوجي
المستشفى الجامعي كليه الطب البشري

Patient's Data

Patient Name: عفاف احمد السيد

Age: 51 y Gender: Female

Referred By Prof. Dr. أ.د سعيد البحيري

Nature of the specimen: Tru-Cut Biopsy

Clinical Diagnosis: Left Breast Mass

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- Positive for lymphovascular invasion.
- Mild peritumoral lymphocytosis
- No Intraductal components were detected.

Diagnosis:

Left Breast Mass: Tru-Cut Biopsy

Features of Invasive Duct Carcinoma, Grade III

For Immunophenotyping (ER, PR, HER/2 NEU & KI67).

Eman A. Abd Elmaogod

Abla S. Mahmoud

Patient name: Afaf Ahmed
Date: 4/1/2025
Prof.Dr. Samar Khamis

US Examination Of Both Breasts Revealed:

- ACR 2: Mixed fibro-fatty-glandular breast parenchyma.
- **An ill-defined hyperdense lesion is seen at the UOQ of the left breast corresponding by US to an ill defined globular shaped hetrogenous hypoechoic lesion is seen at 2 o'clock surrounded by echogenic halo and causing posterior shadowing. It measures about 44 x 32 x 19 mm. associated mid edematous changes manifested by parenchymal coarsening and skin thickening are noted.**
- **The left axillary region shows multiple enlarged lymph nodes showing increased cortical thickness, and yet preserved fatty hila, the largest measures about 22 x 12 mm.**
- No right breast solid or cystic lesions.
- Normal central ducts system.
- Normal right skin thickness and contour as well as nipple areola complexes.
- Right non specific axillary lymph nodes showing preserved fatty hila.

Opinion:

- **Left breast UOQ suspicious lesion as described (BIRADS IV); for histopathological correlation.**

Best regards,
Dr. Esraa Hatem

***BIRADS** scoring is the **breast** imaging and reporting data system for classifying the radiological diagnosis of **breast** lesions:*

BIRADS 0: Needs further investigations.
BIRADS 1: Normal findings.
BIRADS 2: Benign.
BIRADS 3: Probably benign.
BIRADS 4: Probably malignant.
BIRADS 5: Malignant.
BIRADS 6: Proven pathologically malignant

Dr.Esraa Hatem
Radiology specialist

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
إدارة الأشعة

Patient Name: Afaf Ahmed Al Sayed Elsawy	Date : 13/01/2025
Exam : C.T ABDOMEN &PELVIS	Age : 52 Year(s)
Medical ID: 108/2025	Pat. ID: 96847

NON CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

Findings:

I Chest:-

- Left breast retroareolar irregular spiculated mass lesion seen measuring 4 X 5 cm seen reaching the nipple and thickened skin.
- Pathologically enlarged left axillary level I and II lymph nodes the largest is seen measuring 1.3 X 1.9 cm.
- Enlarged left lower deep cervical lymph nodes reaching 1.5 cm.
- No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- Enlarged mediastinal lymph nodes the largest is prevascular measuring 25 X 12 mm..
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II Abdomen & pelvis:

- The liver is of average size with bilobar hypodense hepatic focal lesions the largest is at left lobe segment II measuring 1.2 X 1.7 cm.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

***Scanned skeleton:** L5 vertebral body tiny lytic lesion.

Opinion:

- **Locally advanced left breast malignant process with pathological ipsilateral axillary lymphadenopathy.**
- **Mediastinal and left lower deep cervical lymph nodes worrisome of metastatic deposits.**

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Afaf Ahmed Al Sayed Esawy	Date : 13/01/2025
Exam : C.T ABDOMEN &PELVIS	Age : 52 Year(s)
Medical ID: 108/2025	Pat. ID: 96847

- **Bilobar hepatic focal lesions for triphasic CT assessment.**
- **L5 tiny lytic lesion.**
- **For correlation with the unavailable previous studies.**

Dr. Mohamed Mohsen, M.D.

Patient Name	الأستاذ / عبدالله احمد السيد عيسوي	Request No.	13003629
Patient No.	272082901024 69	Request Date	13/01/2025
Path No.	SR-105/25	Verification Date	29/01/2025
Sex	Female	Financial Category	اورام دار السلام - دفعة الدولة
Age	52 Y	Department	Out Patient
Referring Doc.		Registered by	Mrs.Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/mocerate and within < 10%of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature
Prof. Dr.Gamal Abdelatty
Dr Heba Hassan

Verified by :
Dr.Toka Medhat

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Scanned with CamScanner

Patient Name	: الأستاذه / هطف احمد السيد عيسوي	Request No.	: 13003629
Patient No.	: 27206260102469	Request Date	: 13/01/2025
Path No.	: SR-105/25	Verification Date	: 29/01/2025
Sex	: Female	Financial Category	: اورام دار السلام - نقابة الشرطة
Age	: 52 Y	Department	: Out Patient
Referreing Doc.	:	IHC No.	:
		Registered by	: Mrs.Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast mass.
Nature of Specimen: Slide revision

Technique:
- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) stained by rabbit monoclonal antibodies using Ultraview HRP DAB as chromogen and hematoxylin as counter-stain. The sections were treated against: ER clone (1E2), PR clone (5P1), Her2 clone (4B5) and Ki67 clone (30-9).

Results:
* IPT (Positive control showed appropriate reactivity):
- ER: No nuclear reaction in tumor cells (Allred score 0/8).
- PR: No nuclear reaction in tumor cells (Allred score 0/8).
- Her2: Strong complete membrane reaction in>10 % tumor cells (score 3+).
- Ki67: 10% nuclear reaction of tumor cells.

Conclusion:
Left breast cancer, slide revision:
Invasive duct carcinoma, Grade II, with focal DCIS.
IPT:
- ER: Negative (score 0/8).
- PR: Negative (score 0/8).
- Her2: Positive (score 3+).
- Ki67: Positive in 10% of tumor cells.

* Scoring system: "ER, PR"
ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0 no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and

Verified by :
Dr.Toka Medhat

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Patient Name	:الأستاذه / عفاف احمد السيد حجوري	Request No.	:13003629
Patient No.	:27206260102469	Request Date	:13/01/2025
Path No.	:SR-105/25	Verification Date	:21/01/2025
Sex	:Female	Financial Category	:فورام دار السلام - نقابة المرئة
Age	:52 Y	Department	:Out Patient
Refereing Doc :		Registered by	:Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Left breast cancer.
Nature of Specimen: Slide revision.

Gross Examination:
One paraffin block labled by (P012-25) and one H&E slide were received.

Microscopic Examination:
* Histological type: Invasive duct carcinoma.
* Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+2) Grade II.
* DCIS: Focally positive.
* AngioInvasion: Could not be detected within the limits of biopsy received

Diagnosis:
Left breast mass, Slide revision:
Invasive duct carcinoma, Grade II with focal DCIS.
- For further IPT (ER, PR, Her2 and Ki67).

Signature
Prof. Dr. Lubna Omer
Dr. Menna Nabil

Verified by :
Dr.Heba Hasan

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
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Scanned with CamScanner

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

السيدات المصرية هي دايما مصر

MDT FORM 1/2/2025 GM

Name	عناق احمد السيد عيسوى	MRN	٢٠٢٥١٠٨
Age	52	ID	٢٧٠٢٠٠٤٠٢٢٦
Menopausal status		Phone	٠١٢٨١٤٨٤٠٦٢
NRS SCORE		Marital status	
BMI		Family history	
CO morbidity		REASON OF MDT	NEW

Clinical Data

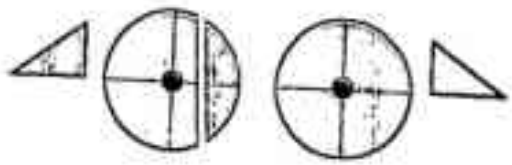
BX: IDC

ER:	PR:	HER 2:	KI67:
0	0	+3	

Sonomammography:

Mets Workup:

R L



TNM

- Mediastinal L.N
- Bilobar F.L

Recommendations:

MDT OUTCOME & PLAN

1/2/2025 → for - 1st line Taxol (herceptin IV (3C)
For Re evaluate.
- baseline CT chest, PA etcontra

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

جوستافروسي

مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

البنت المصرية هي صحة مصر

MDT FORM / /٢٠ G

Name	كفاني محمد السيد عويس	MRN	
		ID	
Age	52	Phone	
Menopausal status		Marital status	
NRS SCORE		Family history	
BMI		REASON OF MDT	
CO morbidity			

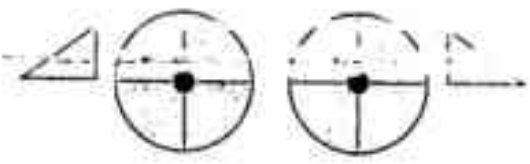
Clinical Data

BX: IDC CD

ER:	PR:	HER 2:	KI 67:
0	0	+3	

Sonomammography:

R L



Mets Workup: Metastatic Rt breast Ca to bone, liver, L.N. Received Taxol / Herceptin x20 week CT.CAP 21/5/20 → RD of multiple lesion

TNM

Recommendations: RD of axillary L.N. For MDT for next plan of Management

21/7/20

MDT OUTCOME & PLAN

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

GUSTAVE ROUSSY
INTERNATIONAL

Patient Name: Afaf Ahmed Al Sayed Elsawy	Date : 21/07/2025
Exam : Axillary u/s	Age : 53 Year(s)
Medical ID: 108/2025	Pat. ID: 96847

Soft tissue ultrasound examination of right axilla

- *Few enlarged axillary LNs at level I, the largest one is measuring 18x7mm showing relatively thickened cortex 3.5mm? reactionary for follow up*
- *Normal skin thickness and subcutaneous tissues*
- *Normal vascular structure*

- Signature Dr . Nermien Ramadan
- 21/07/2025 12:35 PM

Patient Name: Afaf Ahmed Al Sayed Eisawy	Date : 07/08/2025
Exam : Item 1	Age : 53 Year(s)
Medical ID: DAR ELSALAM HOSPITAL^BRAIN	Pat. ID: 96847

MRI EXAMINATION OF THE BRAIN

MR Technique:

- Axial T1, T2 and FLAIR WI's
- Sagittal T1 WI
- Coronal T2 WI
- Axial DWI and ADC
- Post contrast T1 series

MR findings:

- W dened cortical sulci, basal cisterns, cerebellar folia and extra-axial CSF spaces.
- Mild bilateral symmetrical dilatation of the supra-tentorial ventricular system
- Nc cerebral, cerebellar or brain stem areas of altered MR signal.
- Nc midline shift or deformity.
- No extra axial collections or masses.
- No cerebello pontine angle masses.
- No gross posterior fossa abnormalities.

Opinion:

- **Mild age matched brain involutional changes.**

Dr.Zeyad El-Sayed,FEBR

GUSTAVE ROUSSY INTERNATIONAL EGYPT

Patient Name: Afaf Ahmed Al Sayed Eisawy

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

Findings:

Compared to 21/5/2025

I Chest:-

- **Stationary course** of the:
  1. Left breast retroareolar irregular spiculated mass lesion seen measuring 2 X 1.28 cm compared to 4 X 5 cm seen reaching the nipple and thickened skin.
  2. Pathologically enlarged left axillary level I and II lymph nodes being subcm in current study.
- **Stationary course** of one of the right axillary level I lymph nodes reaching 9 X 12 mm.
- Non appreciated forenoted enlarged left lower deep cervical lymph nodes in current exam field.
- No sizable pulmonary nodules (apart from currently noted left basal 3 mm nodule), masses, areas of consolidation or cavitary lesions.
- **Stationary course** of the enlarged mediastinal lymph nodes being subcm.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.



II Abdomen & pelvis:

- The liver is of average size with regressive course of the bilobar hypodense hepatic focal lesions being hardly appreciated in current non contrast study.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

***Scanned skeleton:** more sclerosis L5 vertebral body tiny lytic lesion suggesting favorable response.

Stable L3 sclerotic lesion mostly bone island.

Opinion:

- **Stationary course since the last study.**

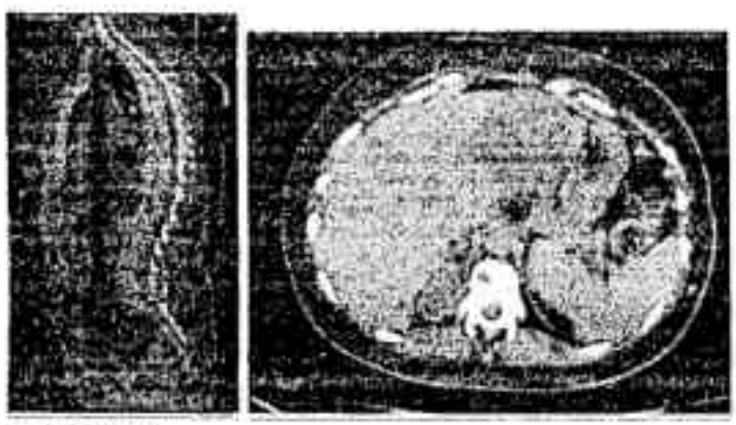
Date 12

Dr. AYA GABR

Name : Afaf El Sayed
Date : 20-7-2025

MULTI-SLICE NON CONTRAST CT SCAN OF THE ABDOMEN AND PELVIS: - PRIMARY REPORT

- ***Known case of right breast cancer. Compared to the previous study dated 5-3-2025, the current study revealed:***



Findings:

- The liver is still seen mildly enlarged (16 cm) with homogenous CT texturestill showing hypodense rather cystic focal lesion occupying segment II and III currently measures about 8x7.8x8.5 cm (compared to 8.2x7.5x7.5 cm), showing thin septations.
- No intra-hepatic or extra-hepatic biliary radicle dilatation.
- Normal size (11.5 cm) and parenchymal texture of the spleen with no sizable focal lesions seen. Still noted two splenules.
- Normal size, parenchymal thickness and density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas and both supra renal glands.
- Still noted multiple subcentemetric abdominal lymph nodes (para-aortic, mesenteric, right common iliac, left external iliac and bilateral inguinal).
- No ascites.
- Average filling of the urinary bladder with no definite stones or masses.
- No gross adnexal lesions.
- Still noted multiple axial and appendicular skeleton lytic lesions with no associated soft tissue component or structural vertebral collapse.

Ministry of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Afaf Ahmed Al Sayed Eisawy	Date : 21/05/2025
Exam : CT CAP(CT Chest Abd. Pelvis)	Age : 53 Year(s)
Medical ID: 108/2025	Pat. ID: 96847

NON CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

Findings:

Compared to 13/1/2025

I Chest:-

- **Regressive course** of the:
  1. Left breast retroareolar irregular spiculated mass lesion seen measuring 2 X 1.28 cm compared to 4 X 5 cm seen reaching the nipple and thickened skin.
  2. Pathologically enlarged left axillary level I and II lymph nodes being subcm in current study.
- **Increase in size** of one of the right axillary level I lymph nodes reaching 9 X 12 mm.
- Non appreciated forenoted enlarged left lower deep cervical lymph nodes in current exam field.
- No sizable pulmonary nodules (apart from currently noted left basal 3 mm nodule), masses, areas of consolidation or cavitary lesions.
- **Regressive course** of the enlarged mediastinal lymph nodes being subcm.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II Abdomen & pelvis:

- The liver is of average size with regressive course of the bilobar hypodense hepatic focal lesions being hardly appreciated in current non contrast study.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

***Scanned skeleton:** **more sclerosis** L5 vertebral body tiny lytic lesion suggesting favorable response.
Stable L3 sclerotic lesion mostly bone island.

Opinion:

- **Increase in size of one of the right axillary lymph nodes for sonographic assessment.**
- **Overall regressive course as described.**

Dr. Mohamed Mohsen, M.D.

Patient Name: Afaf Ahmed Al Sayed Eisawy	Date : 21/07/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 53 Year(s)
Medical ID: 108/2025 1720 1000	Pat. ID: 96847

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST PRIMARY REPORT

- ***Known case of right breast cancer. Compared to the previous study dated 5-3-2025. the current study revealed:***



Findings:

- Right breast operative bed still shows an ill-defined soft tissue density resting over the chest wall muscles measuring 3x1.5 cm for clinical and SM correlation.
- Still noted few subcentimetric left axillary lymph nodes.
- Still noted ubcentimetric left supra-clavicular lymph node.
- Still noted right lower lung lobe posterior segment tiny partially calcified pulmonary nodule about 4 mm.
- Still noted right upper and middle lung lobes subpleural reticulations and faint ground glass opacities, likely post radiation sequel.
- Left basal atelectatic bands are still noted.
- No sizable hilar or mediastinal lymph nodes regarding this non contrast study.
- Patent tracheo- bronchial tree.
- Average size of the heart.
- No evidence of pleural or pericardial effusion.

Patient Name: Afaf Ahmed Al Sayed Elsawy	Date : 21/07/2025
Exam : CT CAP+C (CT Chest Abc. Pelvis)	Age : 53 Year(s)
Medical ID: 108/2025 1720 1000	Pat. ID: 96847

OPINION:

- ***Stationary course of the disease process.***

Marina Essam, MSc

مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

MDT FORM 1/2/2025 GM

Name	عفاف احمد السيد عيسوى	MRN	C٤٥١٠٨
Age	52	ID	٢٧٠٤٠٠٠٤٢
Menopausal status		Phone	٠١٤٥١٤٥٤٦٢
NRS SCORE		Marital status	
BMI	[redacted]	Family history	
CO morbidity		REASON OF MDT	NEW

Clinical Data

BX: IDC

ER:	PR:	HER 2:	KI67:
C	0	+3	

Sonomammography:

Mets Workup:

R L

TNM
- Mediastinal L.N
- B.lobar F.L

Recommendations:

MDT OUTCOME & PLAN

1/2/2025 → Fit - 1st line Taxol (herceptin IV) (SC)
Then Letrozole.
- baseline CT chest, FA etcetera

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

جوستافروسي

مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

البنك المصرية هي صحة مصر

MDT FORM / /20 G

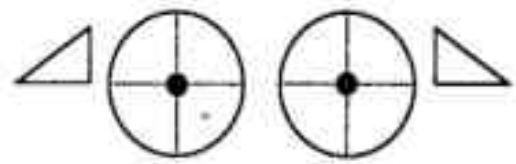
Name	دعاء أحمد السيد	MRN	10812025
		ID	
Age	52	Phone	
Menopausal status		Marital status	
NRS SCORE		Family history	
BMI		REASON OF MDT	
CO morbidity			

Clinical Data

BX: IDC GIII

ER:	PR:	HER 2:	KI67:
0/8	0/8	+3	

R L



Sonomammography:

Mets Workup: Mets breast Cancer to Liver & LN TNM
received 1st line taxol weekly + Herceptin
last CTCAP → SD. (Cycle 4 to be)

Recommendations: for MDT. i.e. BS Cycle 4
5-2025
Dr 31/7/2025. Consider 2nd line Xeloda-Tykerb

MDT OUTCOME & PLAN

SIGNATURE					
MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

Patient Name	: الأستاذه / عفاف احمد السيد عيسوي	Request No.	: 13003629
Patient No.	: 27206260102469	Request Date	: 13/01/2025
Path No.	: SR-105/25	Verification Date	: 21/01/2025
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 52 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Left breast cancer.
Nature of Specimen: Slide revision.

Gross Examination:
One paraffin block labled by (P012-25) and one H&E slide were received.

Microscopic Examination:
* Histological type: Invasive duct carcinoma.
* Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+2) Grade II.
* DCIS: Focally positive.
* Angioinvasion: Could not be detected within the limits of biopsy received

Diagnosis:
Left breast mass, Slide revision:
Invasive duct carcinoma, Grade II with focal DCIS.
- For further IPT (ER, PR, Her2 and Ki67).

Signature
Prof. Dr. Lubna Omar
Dr. Menna Nabil

ERM036748357

Verified by :
Dr.Heba Hasan

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

IEYA SCAN

مركز
هي للأشعة

NAME : Afaf Ahmad
DATE: 8/1/2025
PROF .DR; SaedAlbehery

Bilateral MAMMOGRAM and HIGH RESOLUTION ULTRASOUND EXAMINATION

- **ACR B breast parenchyma**
- **An ill-defined spiculated radio-dense lesion is seen at left breast at upper outer quadrant ,associated diffuse increased skin thickness is seen**
- **Complementary u/S reveals: An ill-defined hypoechoic lesion is seen at the upper outer quadrant of the left breast at 12-3 o'clock measuring about 35x25mm ,few internal vascularity is noted by color Doppler study .The lesion is seen surrounded by echogenic breast parenchyma (desmoplastic reaction).multiple tiny echogenic foci seen inside likely microcalcification**
- **No dilated lactiferous ducts**
- Norma right skin thickness and contour.
- Normal nipple areola complex.
- **Multiple enlarged left axillary lymph nodes, the largest measures about 29x25mm ,some show effaced fatty hila**

OPINION:

- ***Left breast upper outer quadrant soft tissue lesions likely malignant associated with pathological left axillary lymph nodes for histopathological correlation (BIRADS 5)***

Much obliged,
DR .Eman AbdelRahman ,MD

01119598889 - 01005425312 0822167074
بني سويف - ش احمد عرابي برج زيادة 2 - أمام قصر الثقافة
بني سويف - أمام أجيال للملابس

Patient Name: AFAF AHMED ALSAYED

Patient ID: 32**154

Gems
Nuclear Imaging Environment

Study Name: BONE SCAN

Date & Time: 2/2/2015

Manufacturer Model: VARICAM





فيجا لاب

لتحاليل الخلايا والأنسجة والاورام

ا.د/ عبله سيد محمود عبد العليم
دكتوراه واستشاري الباثولوجي
المستشفى الجامعي كليه الطب البشري

ا.د/ إيمان أحمد عبد الموجود
دكتوراه واستشاري الباثولوجي
المستشفى الجامعي كليه الطب البشري

Patient's Data

Patient Name: عفاف أحمد السيد

Age: 51 y Gender: Female

Referred By Prof. Dr. أ.د سعيد البحيري

Nature of the specimen: Tru-Cut Biopsy

Clinical Diagnosis: Left Breast Mass

Code Number: P0012 -25

Received At : 5- 1 - 2025

Report issued At : 9-1-2025

PATHOLOGY REPORT

Gross Pathology:

Multiple grayish white tissue cores of average length 1 cm, totally submitted.

Histology:

- Examination of the specimen received revealed breast tissue cores infiltrated by malignant epithelial cells displaying moderate degree of anaplasia, arranged in nests, individual cells surrounded by desmoplastic stroma.
- Positive for lymphovascular invasion.
- Mild peritumoral lymphocytosis
- No Intraductal components were detected.

Diagnosis:

Left Breast Mass; Tru-Cut Biopsy

Features of Invasive Duct Carcinoma, Grade III

For Immunophenotyping (ER, PR, HER/2 NEU & KI67).

معمل فيجا لاب للتحاليل الخلايا والأنسجة والأورام

Eman A. Abd Elmaogod

Abla S. Mahmoud

Patient name: Ms. Afaf Ahmad Al Sayed

Referred by: Prof. Dr.

Date: Feb ,03.2021

Tc^{99m}-MDP- Bone scan

Procedure:

- Anterior and posterior whole body scans were acquired 10 minutes and 2 hours post injection of 20 mCi of the tracer.

Findings:

- No detected increased abnormal tracer uptake over the scanned skeleton, to account for serious bone lesions.
- Osteoarthritic changes of shoulder and right knee joints.
- Scoliotic changes of the dorsal spine.

Opinion

- **No scintigraphic evidence of bone deposits throughout the scanned skeleton.**

Mahmoud Shaban Abd Al-Alim, MD, EBNM

European Board of Nuclear Medicine

فيبا لاب

لتحاليل الخلايا والأنسجة والاورام

ا.د/ عبله سيد محمود عبد العليم
دكتوراه واستشاري الباثولوجي
المستشفى الجامعي كليه الطب البشري

ا.د/ إيمان أحمد عبد الموجود
دكتوراه واستشاري الباثولوجي
المستشفى الجامعي كليه الطب البشري

Patient's Data

Patient Name: عفاف احمد السيد

Age: 51 y Gender: Female

Referred By Prof. Dr. أ.د سعيد البحيري

Nature of the specimen: Tru-Cut Biopsy

Clinical Diagnosis: Left Breast Mass

Received At : 5- 1 - 2025

Code Number: P0012 -25

Report issued A : 9-1-2025

PATHOLOGY REPORT

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- Positive for lymphovascular invasion.
- Mild peritumoral lymphocytosis
- No Intraductal components were detected.

Diagnosis:

Left Breast Mass: Tru-Cut Biopsy

Features of Invasive Duct Carcinoma, Grade III

For Immunophenotyping (ER, PR, HER/2 NEU & KI67).

Eman A. Abd Elmaogod

Abla S. Mahmoud

Patient name: Afaf Ahmed
Date: 4/1/2025
Prof.Dr. Samar Khamis

US Examination Of Both Breasts Revealed:

- ACR 2: Mixed fibro-fatty-glandular breast parenchyma.
- **An ill-defined hyperdense lesion is seen at the UOQ of the left breast corresponding by US to an ill defined globular shaped hetrogenous hypoechoic lesion is seen at 2 o'clock surrounded by echogenic halo and causing posterior shadowing. It measures about 44 x 32 x 19 mm. associated mid edematous changes manifested by parenchymal coarsening and skin thickening are noted.**
- **The left axillary region shows multiple enlarged lymph nodes showing increased cortical thickness, and yet preserved fatty hila, the largest measures about 22 x 12 mm.**
- No right breast solid or cystic lesions.
- Normal central ducts system.
- Normal right skin thickness and contour as well as nipple areola complexes.
- Right non specific axillary lymph nodes showing preserved fatty hila.

Opinion:

- **Left breast UOQ suspicious lesion as described (BIRADS IV); for histopathological correlation.**

Best regards,
Dr. Esraa Hatem

***BIRADS** scoring is the **breast** imaging and reporting data system for classifying the radiological diagnosis of **breast** lesions:*

BIRADS 0: Needs further investigations.
BIRADS 1: Normal findings.
BIRADS 2: Benign.
BIRADS 3: Probably benign.
BIRADS 4: Probably malignant.
BIRADS 5: Malignant.
BIRADS 6: Proven pathologically malignant

Dr.Esraa Hatem
Radiology specialist

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
إدارة الأشعة

Patient Name: Afaf Ahmed Al Sayed Elsawy	Date : 13/01/2025
Exam : C.T ABDOMEN &PELVIS	Age : 52 Year(s)
Medical ID: 108/2025	Pat. ID: 96847

NON CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

Findings:

I Chest:-

- Left breast retroareolar irregular spiculated mass lesion seen measuring 4 X 5 cm seen reaching the nipple and thickened skin.
- Pathologically enlarged left axillary level I and II lymph nodes the largest is seen measuring 1.3 X 1.9 cm.
- Enlarged left lower deep cervical lymph nodes reaching 1.5 cm.
- No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- Enlarged mediastinal lymph nodes the largest is prevascular measuring 25 X 12 mm..
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II Abdomen & pelvis:

- The liver is of average size with bilobar hypodense hepatic focal lesions the largest is at left lobe segment II measuring 1.2 X 1.7 cm.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

***Scanned skeleton:** L5 vertebral body tiny lytic lesion.

Opinion:

- **Locally advanced left breast malignant process with pathological ipsilateral axillary lymphadenopathy.**
- **Mediastinal and left lower deep cervical lymph nodes worrisome of metastatic deposits.**

Ministry Of Health
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أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Afaf Ahmed Al Sayed Esawy	Date : 13/01/2025
Exam : C.T ABDOMEN &PELVIS	Age : 52 Year(s)
Medical ID: 108/2025	Pat. ID: 96847

- **Bilobar hepatic focal lesions for triphasic CT assessment.**
- **L5 tiny lytic lesion.**
- **For correlation with the unavailable previous studies.**

Dr. Mohamed Mohsen, M.D.

Patient Name	الأستاذ / عبدالله احمد السيد عيسوي	Request No.	13003629
Patient No.	272082901024 69	Request Date	13/01/2025
Path No.	SR-105/25	Verification Date	29/01/2025
Sex	Female	Financial Category	اورام دار السلام - دفعة الدولة
Age	52 Y	Department	Out Patient
Referring Doc.		Registered by	Mrs.Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/mocerate and within < 10%of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature
Prof. Dr.Gamal Abdelatty
Dr Heba Hassan

Verified by :
Dr.Toka Medhat

غرفة رقم (4) – الدور الأول – مبنى رقم (307) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 307- Nile Corniche - Misr El Qadema - Cairo, Egypt

Scanned with CamScanner

Patient Name	: الأستاذه / هطف احمد السيد عيسوي	Request No.	: 13003629
Patient No.	: 27206260102469	Request Date	: 13/01/2025
Path No.	: SR-105/25	Verification Date	: 29/01/2025
Sex	: Female	Financial Category	: اورام دار السلام - نقابة الشرطة
Age	: 52 Y	Department	: Out Patient
Referreing Doc.	:	IHC No.	:
		Registered by	: Mrs.Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast mass.
Nature of Specimen: Slide revision

Technique:
- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) stained by rabbit monoclonal antibodies using Ultraview HRP DAB as chromogen and hematoxylin as counter-stain. The sections were treated against: ER clone (1E2), PR clone (5P1), Her2 clone (4B5) and Ki67 clone (30-9).

Results:
* IPT (Positive control showed appropriate reactivity):
- ER: No nuclear reaction in tumor cells (Allred score 0/8).
- PR: No nuclear reaction in tumor cells (Allred score 0/8).
- Her2: Strong complete membrane reaction in>10 % tumor cells (score 3+).
- Ki67: 10% nuclear reaction of tumor cells.

Conclusion:
Left breast cancer, slide revision:
Invasive duct carcinoma, Grade II, with focal DCIS.
IPT:
- ER: Negative (score 0/8).
- PR: Negative (score 0/8).
- Her2: Positive (score 3+).
- Ki67: Positive in 10% of tumor cells.

* Scoring system: "ER, PR"
ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0 no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and

Verified by :
Dr.Toka Medhat

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Scanned with CamScanner

Patient Name	:الأستاذه / عفاف احمد السيد حجوري	Request No.	:13003629
Patient No.	:27206260102469	Request Date	:13/01/2025
Path No.	:SR-105/25	Verification Date	:21/01/2025
Sex	:Female	Financial Category	:فورام دار السلام - نقابة المرئة
Age	:52 Y	Department	:Out Patient
Refereing Doc :		Registered by	:Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Left breast cancer.
Nature of Specimen: Slide revision.

Gross Examination:
One paraffin block labled by (P012-25) and one H&E slide were received.

Microscopic Examination:
* Histological type: Invasive duct carcinoma.
* Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+2) Grade II.
* DCIS: Focally positive.
* AngioInvasion: Could not be detected within the limits of biopsy received

Diagnosis:
Left breast mass, Slide revision:
Invasive duct carcinoma, Grade II with focal DCIS.
- For further IPT (ER, PR, Her2 and Ki67).

Signature
Prof. Dr. Lubna Omer
Dr. Menna Nabil

Verified by :
Dr.Heba Hasan

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مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

السيدات المصرية هي دايما مصر

MDT FORM 1/2/2025 GM

Name	عناق احمد السيد عيسوى	MRN	٢٠٢٥١٠٨
Age	52	ID	٢٧٠٢٠٠٤٠٢٢٦
Menopausal status		Phone	٠١٢٨١٤٨٤٠٦٢
NRS SCORE		Marital status	
BMI		Family history	
CO morbidity		REASON OF MDT	NEW

Clinical Data

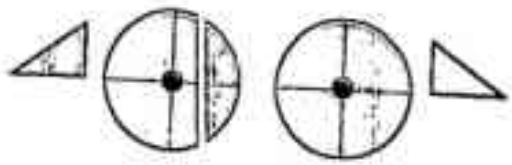
BX: IDC

ER:	PR:	HER 2:	KI67:
0	0	+3	

Sonomammography:

Mets Workup:

R L



TNM

- Mediastinal L.N
- Bilobar F.L

Recommendations:

MDT OUTCOME & PLAN

1/2/2025 → for - 1st line Taxol (herceptin IV (3C)
For Re evaluate.
- baseline CT chest, PA etcontra

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

جوستافروسي

مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

البنت المصرية هي صحة مصر

MDT FORM / /٢٠ G

Name	كفاني محمد السيد عويس	MRN	
		ID	
Age	52	Phone	
Menopausal status		Marital status	
NRS SCORE		Family history	
BMI		REASON OF MDT	
CO morbidity			

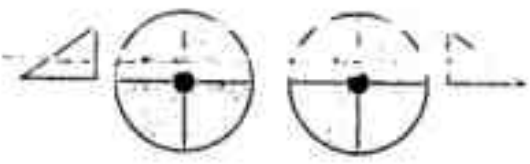
Clinical Data

BX: IDC CD

ER:	PR:	HER 2:	KI 67:
0	0	+3	

Sonomammography:

R L



Mets Workup: Metastatic Rt breast Ca to bone, liver, L.N. Received Taxol / Herceptin x20 week CT.CAP 21/5/20 → RD of multiple lesion

TNM

Recommendations: RD of axillary L.N. For MDT for next plan of Management

21/7/20

MDT OUTCOME & PLAN

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

GUSTAVE ROUSSY
INTERNATIONAL

Patient Name: Afaf Ahmed Al Sayed Elsawy	Date : 21/07/2025
Exam : Axillary u/s	Age : 53 Year(s)
Medical ID: 108/2025	Pat. ID: 96847

Soft tissue ultrasound examination of right axilla

- *Few enlarged axillary LNs at level I, the largest one is measuring 18x7mm showing relatively thickened cortex 3.5mm? reactionary for follow up*
- *Normal skin thickness and subcutaneous tissues*
- *Normal vascular structure*

- Signature Dr . Nermien Ramadan
- 21/07/2025 12:35 PM

Patient Name: Afaf Ahmed Al Sayed Eisawy	Date : 07/08/2025
Exam : Item 1	Age : 53 Year(s)
Medical ID: DAR ELSALAM HOSPITAL^BRAIN	Pat. ID: 96847

MRI EXAMINATION OF THE BRAIN

MR Technique:

- Axial T1, T2 and FLAIR WI's
- Sagittal T1 WI
- Coronal T2 WI
- Axial DWI and ADC
- Post contrast T1 series

MR findings:

- W dened cortical sulci, basal cisterns, cerebellar folia and extra-axial CSF spaces.
- Mild bilateral symmetrical dilatation of the supra-tentorial ventricular system
- Nc cerebral, cerebellar or brain stem areas of altered MR signal.
- Nc midline shift or deformity.
- No extra axial collections or masses.
- No cerebello pontine angle masses.
- No gross posterior fossa abnormalities.

Opinion:

- **Mild age matched brain involutional changes.**

Dr.Zeyad El-Sayed,FEBR

GUSTAVE ROUSSY INTERNATIONAL EGYPT

Patient Name: Afaf Ahmed Al Sayed Eisawy

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

Findings:

Compared to 21/5/2025

I Chest:-

- **Stationary course** of the:
  1. Left breast retroareolar irregular spiculated mass lesion seen measuring 2 X 1.28 cm compared to 4 X 5 cm seen reaching the nipple and thickened skin.
  2. Pathologically enlarged left axillary level I and II lymph nodes being subcm in current study.
- **Stationary course** of one of the right axillary level I lymph nodes reaching 9 X 12 mm.
- Non appreciated forenoted enlarged left lower deep cervical lymph nodes in current exam field.
- No sizable pulmonary nodules (apart from currently noted left basal 3 mm nodule), masses, areas of consolidation or cavitary lesions.
- **Stationary course** of the enlarged mediastinal lymph nodes being subcm.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.



II Abdomen & pelvis:

- The liver is of average size with regressive course of the bilobar hypodense hepatic focal lesions being hardly appreciated in current non contrast study.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

***Scanned skeleton:** more sclerosis L5 vertebral body tiny lytic lesion suggesting favorable response.

Stable L3 sclerotic lesion mostly bone island.

Opinion:

- **Stationary course since the last study.**

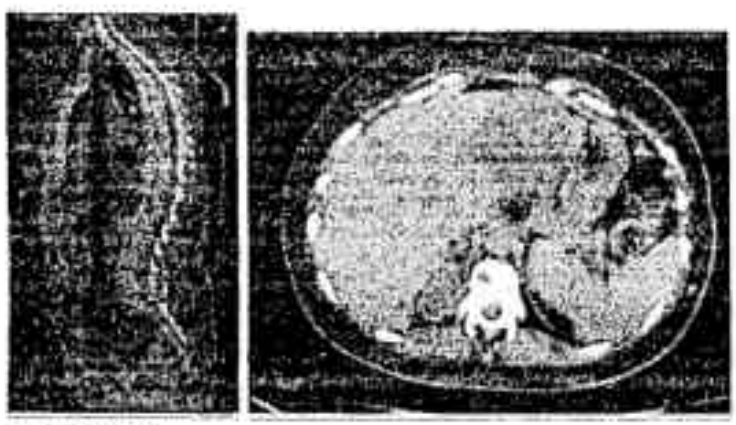
Date 12

Dr. AYA GABR

Name : Afaf El Sayed
Date : 20-7-2025

MULTI-SLICE NON CONTRAST CT SCAN OF THE ABDOMEN AND PELVIS: - PRIMARY REPORT

- ***Known case of right breast cancer. Compared to the previous study dated 5-3-2025, the current study revealed:***



Findings:

- The liver is still seen mildly enlarged (16 cm) with homogenous CT texturestill showing hypodense rather cystic focal lesion occupying segment II and III currently measures about 8x7.8x8.5 cm (compared to 8.2x7.5x7.5 cm), showing thin septations.
- No intra-hepatic or extra-hepatic biliary radicle dilatation.
- Normal size (11.5 cm) and parenchymal texture of the spleen with no sizable focal lesions seen. Still noted two splenules.
- Normal size, parenchymal thickness and density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas and both supra renal glands.
- Still noted multiple subcentemetric abdominal lymph nodes (para-aortic, mesenteric, right common iliac, left external iliac and bilateral inguinal).
- No ascites.
- Average filling of the urinary bladder with no definite stones or masses.
- No gross adnexal lesions.
- Still noted multiple axial and appendicular skeleton lytic lesions with no associated soft tissue component or structural vertebral collapse.

Ministry of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Afaf Ahmed Al Sayed Eisawy	Date : 21/05/2025
Exam : CT CAP(CT Chest Abd. Pelvis)	Age : 53 Year(s)
Medical ID: 108/2025	Pat. ID: 96847

NON CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

Findings:

Compared to 13/1/2025

I Chest:-

- **Regressive course** of the:
  1. Left breast retroareolar irregular spiculated mass lesion seen measuring 2 X 1.28 cm compared to 4 X 5 cm seen reaching the nipple and thickened skin.
  2. Pathologically enlarged left axillary level I and II lymph nodes being subcm in current study.
- **Increase in size** of one of the right axillary level I lymph nodes reaching 9 X 12 mm.
- Non appreciated forenoted enlarged left lower deep cervical lymph nodes in current exam field.
- No sizable pulmonary nodules (apart from currently noted left basal 3 mm nodule), masses, areas of consolidation or cavitary lesions.
- **Regressive course** of the enlarged mediastinal lymph nodes being subcm.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II Abdomen & pelvis:

- The liver is of average size with regressive course of the bilobar hypodense hepatic focal lesions being hardly appreciated in current non contrast study.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

***Scanned skeleton:** **more sclerosis** L5 vertebral body tiny lytic lesion suggesting favorable response.
Stable L3 sclerotic lesion mostly bone island.

Opinion:

- **Increase in size of one of the right axillary lymph nodes for sonographic assessment.**
- **Overall regressive course as described.**

Dr. Mohamed Mohsen, M.D.

Patient Name: Afaf Ahmed Al Sayed Eisawy	Date : 21/07/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 53 Year(s)
Medical ID: 108/2025 1720 1000	Pat. ID: 96847

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST PRIMARY REPORT

- ***Known case of right breast cancer. Compared to the previous study dated 5-3-2025. the current study revealed:***



Findings:

- Right breast operative bed still shows an ill-defined soft tissue density resting over the chest wall muscles measuring 3x1.5 cm for clinical and SM correlation.
- Still noted few subcentimetric left axillary lymph nodes.
- Still noted ubcentimetric left supra-clavicular lymph node.
- Still noted right lower lung lobe posterior segment tiny partially calcified pulmonary nodule about 4 mm.
- Still noted right upper and middle lung lobes subpleural reticulations and faint ground glass opacities, likely post radiation sequel.
- Left basal atelectatic bands are still noted.
- No sizable hilar or mediastinal lymph nodes regarding this non contrast study.
- Patent tracheo- bronchial tree.
- Average size of the heart.
- No evidence of pleural or pericardial effusion.

Patient Name: Afaf Ahmed Al Sayed Elsawy	Date : 21/07/2025
Exam : CT CAP+C (CT Chest Abc. Pelvis)	Age : 53 Year(s)
Medical ID: 108/2025 1720 1000	Pat. ID: 96847

OPINION:

- ***Stationary course of the disease process.***

Marina Essam, MSc

مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

MDT FORM 1/2/2025 GM

Name	عفاف احمد السيد عيسوى	MRN	C٤٥١٠٨
Age	52	ID	٢٧٠٤٠٠٠٤٢
Menopausal status		Phone	٠١٤٥١٤٥٤٦٢
NRS SCORE		Marital status	
BMI	[redacted]	Family history	
CO morbidity		REASON OF MDT	NEW

Clinical Data

BX: IDC

ER:	PR:	HER 2:	KI67:
C	0	+3	

Sonomammography:

Mets Workup:

R L

TNM
- Mediastinal L.N
- B.lobar F.L

Recommendations:

MDT OUTCOME & PLAN

1/2/2025 → Fit - 1st line Taxol (herceptin IV) (SC)
Then Letrozole.
- baseline CT chest, FA etcetera

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

جوستافروسي

مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

البنك المصرية هي صحة مصر

MDT FORM / /20 G

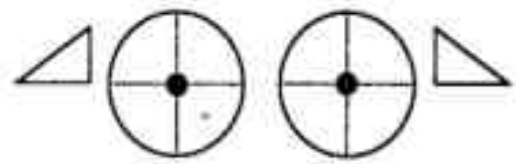
Name	دعاء أحمد السيد	MRN	10812025
		ID	
Age	52	Phone	
Menopausal status		Marital status	
NRS SCORE		Family history	
BMI		REASON OF MDT	
CO morbidity			

Clinical Data

BX: IDC GIII

ER:	PR:	HER 2:	KI67:
0/8	0/8	+3	

R L



Sonomammography:

Mets Workup: Mets breast Cancer to Liver & LN TNM
received 1st line taxol weekly + Herceptin
last CTCAP → SD. (Cycle 4 to be)

Recommendations: for MDT. i.e. BS Cycle 4
5-2025
Dr 31/7/2025. Consider 2nd line Xeloda-Tykerb

MDT OUTCOME & PLAN

SIGNATURE					
MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

Patient Name	: الأستاذه / عفاف احمد السيد عيسوي	Request No.	: 13003629
Patient No.	: 27206260102469	Request Date	: 13/01/2025
Path No.	: SR-105/25	Verification Date	: 21/01/2025
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 52 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Left breast cancer.
Nature of Specimen: Slide revision.

Gross Examination:
One paraffin block labled by (P012-25) and one H&E slide were received.

Microscopic Examination:
* Histological type: Invasive duct carcinoma.
* Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+2) Grade II.
* DCIS: Focally positive.
* Angioinvasion: Could not be detected within the limits of biopsy received

Diagnosis:
Left breast mass, Slide revision:
Invasive duct carcinoma, Grade II with focal DCIS.
- For further IPT (ER, PR, Her2 and Ki67).

Signature
Prof. Dr. Lubna Omar
Dr. Menna Nabil

ERM036748357

Verified by :
Dr.Heba Hasan

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

IEYA SCAN

مركز
هي للأشعة

NAME : Afaf Ahmad
DATE: 8/1/2025
PROF .DR; SaedAlbehery

Bilateral MAMMOGRAM and HIGH RESOLUTION ULTRASOUND EXAMINATION

- **ACR B breast parenchyma**
- **An ill-defined spiculated radio-dense lesion is seen at left breast at upper outer quadrant ,associated diffuse increased skin thickness is seen**
- **Complementary u/S reveals: An ill-defined hypoechoic lesion is seen at the upper outer quadrant of the left breast at 12-3 o'clock measuring about 35x25mm ,few internal vascularity is noted by color Doppler study .The lesion is seen surrounded by echogenic breast parenchyma (desmoplastic reaction).multiple tiny echogenic foci seen inside likely microcalcification**
- **No dilated lactiferous ducts**
- Norma right skin thickness and contour.
- Normal nipple areola complex.
- **Multiple enlarged left axillary lymph nodes, the largest measures about 29x25mm ,some show effaced fatty hila**

OPINION:

- ***Left breast upper outer quadrant soft tissue lesions likely malignant associated with pathological left axillary lymph nodes for histopathological correlation (BIRADS 5)***

Much obliged,
DR .Eman AbdelRahman ,MD

01119598889 - 01005425312 0822167074
بني سويف - ش احمد عرابي برج زيادة 2 - أمام قصر الثقافة
بني سويف - أمام أجيال للملابس

Patient Name: AFAF AHMED ALSAYED

Patient ID: 32**154

Gems
Nuclear Imaging Environment

Study Name: BONE SCAN

Date & Time: 2/2/2015

Manufacturer Model: VARICAM





فيجا لاب

لتحاليل الخلايا والأنسجة والاورام

ا.د/ عبله سيد محمود عبد العليم
دكتوراه واستشاري الباثولوجي
المستشفى الجامعي كليه الطب البشري

ا.د/ إيمان أحمد عبد الموجود
دكتوراه واستشاري الباثولوجي
المستشفى الجامعي كليه الطب البشري

Patient's Data

Patient Name: عفاف أحمد السيد

Age: 51 y Gender: Female

Referred By Prof. Dr. أ.د سعيد البحيري

Nature of the specimen: Tru-Cut Biopsy

Clinical Diagnosis: Left Breast Mass

Code Number: P0012 -25

Received At : 5- 1 - 2025

Report issued At : 9-1-2025

PATHOLOGY REPORT

Gross Pathology:

Multiple grayish white tissue cores of average length 1 cm, totally submitted.

Histology:

- Examination of the specimen received revealed breast tissue cores infiltrated by malignant epithelial cells displaying moderate degree of anaplasia, arranged in nests, individual cells surrounded by desmoplastic stroma.
- Positive for lymphovascular invasion.
- Mild peritumoral lymphocytosis
- No Intraductal components were detected.

Diagnosis:

Left Breast Mass; Tru-Cut Biopsy

Features of Invasive Duct Carcinoma, Grade III

For Immunophenotyping (ER, PR, HER/2 NEU & KI67).

معمل فيجا لاب للتحاليل الخلايا والأنسجة والأورام

Eman A. Abd Elmaogod

Abla S. Mahmoud

Patient name: Ms. Afaf Ahmad Al Sayed

Referred by: Prof. Dr.

Date: Feb ,03.2021

Tc^{99m}-MDP- Bone scan

Procedure:

- Anterior and posterior whole body scans were acquired 10 minutes and 2 hours post injection of 20 mCi of the tracer.

Findings:

- No detected increased abnormal tracer uptake over the scanned skeleton, to account for serious bone lesions.
- Osteoarthritic changes of shoulder and right knee joints.
- Scoliotic changes of the dorsal spine.

Opinion

- **No scintigraphic evidence of bone deposits throughout the scanned skeleton.**

Mahmoud Shaban Abd Al-Alim, MD, EBNM

European Board of Nuclear Medicine

Patient name: Afaf Ahmed
Date: 4/1/2025
Prof.Dr. Samar Khamis

US Examination Of Both Breasts Revealed:

- ACR 2: Mixed fibro-fatty-glandular breast parenchyma.
- **An ill-defined hyperdense lesion is seen at the UOQ of the left breast corresponding by US to an ill defined globular shaped hetrogenous hypoechoic lesion is seen at 2 o'clock surrounded by echogenic halo and causing posterior shadowing. It measures about 44 x 32 x 19 mm. associated mid edematous changes manifested by parenchymal coarsening and skin thickening are noted.**
- **The left axillary region shows multiple enlarged lymph nodes showing increased cortical thickness, and yet preserved fatty hila, the largest measures about 22 x 12 mm.**
- No right breast solid or cystic lesions.
- Normal central ducts system.
- Normal right skin thickness and contour as well as nipple areola complexes.
- Right non specific axillary lymph nodes showing preserved fatty hila.

Opinion:

- **Left breast UOQ suspicious lesion as described (BIRADS IV); for histopathological correlation.**

Best regards,
Dr. Esraa Hatem

***BIRADS** scoring is the **breast** imaging and reporting data system for classifying the radiological diagnosis of **breast** lesions:*

BIRADS 0: Needs further investigations.
BIRADS 1: Normal findings.
BIRADS 2: Benign.
BIRADS 3: Probably benign.
BIRADS 4: Probably malignant.
BIRADS 5: Malignant.
BIRADS 6: Proven pathologically malignant

Dr.Esraa Hatem
Radiology specialist

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

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أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
إدارة الأشعة

Patient Name: Afaf Ahmed Al Sayed Elsawy	Date : 13/01/2025
Exam : C.T ABDOMEN &PELVIS	Age : 52 Year(s)
Medical ID: 108/2025	Pat. ID: 96847

NON CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

Findings:

I Chest:-

- Left breast retroareolar irregular spiculated mass lesion seen measuring 4 X 5 cm seen reaching the nipple and thickened skin.
- Pathologically enlarged left axillary level I and II lymph nodes the largest is seen measuring 1.3 X 1.9 cm.
- Enlarged left lower deep cervical lymph nodes reaching 1.5 cm.
- No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- Enlarged mediastinal lymph nodes the largest is prevascular measuring 25 X 12 mm..
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II Abdomen & pelvis:

- The liver is of average size with bilobar hypodense hepatic focal lesions the largest is at left lobe segment II measuring 1.2 X 1.7 cm.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

***Scanned skeleton:** L5 vertebral body tiny lytic lesion.

Opinion:

- **Locally advanced left breast malignant process with pathological ipsilateral axillary lymphadenopathy.**
- **Mediastinal and left lower deep cervical lymph nodes worrisome of metastatic deposits.**

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مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Afaf Ahmed Al Sayed Esawy	Date : 13/01/2025
Exam : C.T ABDOMEN &PELVIS	Age : 52 Year(s)
Medical ID: 108/2025	Pat. ID: 96847

- **Bilobar hepatic focal lesions for triphasic CT assessment.**
- **L5 tiny lytic lesion.**
- **For correlation with the unavailable previous studies.**

Dr. Mohamed Mohsen, M.D.

Patient Name	الأستاذ / عبدالله احمد السيد عيسوي	Request No.	13003629
Patient No.	272082901024 69	Request Date	13/01/2025
Path No.	SR-105/25	Verification Date	29/01/2025
Sex	Female	Financial Category	اورام دار السلام - دفعة الدولة
Age	52 Y	Department	Out Patient
Referring Doc.		Registered by	Mrs.Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/mocerate and within < 10%of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature
Prof. Dr.Gamal Abdelatty
Dr Heba Hassan

Verified by :
Dr.Toka Medhat

غرفة رقم (4) – الدور الأول – مبنى رقم (307) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 307- Nile Corniche - Misr El Qadema - Cairo, Egypt

Scanned with CamScanner

Patient Name	: الأستاذه / هطف احمد السيد عيسوي	Request No.	: 13003629
Patient No.	: 27206260102469	Request Date	: 13/01/2025
Path No.	: SR-105/25	Verification Date	: 29/01/2025
Sex	: Female	Financial Category	: اورام دار السلام - نقابة الشرطة
Age	: 52 Y	Department	: Out Patient
Referreing Doc.	:	IHC No.	:
		Registered by	: Mrs.Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast mass.
Nature of Specimen: Slide revision

Technique:
- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) stained by rabbit monoclonal antibodies using Ultraview HRP DAB as chromogen and hematoxylin as counter-stain. The sections were treated against: ER clone (1E2), PR clone (5P1), Her2 clone (4B5) and Ki67 clone (30-9).

Results:
* IPT (Positive control showed appropriate reactivity):
- ER: No nuclear reaction in tumor cells (Allred score 0/8).
- PR: No nuclear reaction in tumor cells (Allred score 0/8).
- Her2: Strong complete membrane reaction in>10 % tumor cells (score 3+).
- Ki67: 10% nuclear reaction of tumor cells.

Conclusion:
Left breast cancer, slide revision:
Invasive duct carcinoma, Grade II, with focal DCIS.
IPT:
- ER: Negative (score 0/8).
- PR: Negative (score 0/8).
- Her2: Positive (score 3+).
- Ki67: Positive in 10% of tumor cells.

* Scoring system: "ER, PR"
ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0 no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and

Verified by :
Dr.Toka Medhat

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Scanned with CamScanner

Patient Name	:الأستاذه / عفاف احمد السيد حجوري	Request No.	:13003629
Patient No.	:27206260102469	Request Date	:13/01/2025
Path No.	:SR-105/25	Verification Date	:21/01/2025
Sex	:Female	Financial Category	:فورام دار السلام - نقابة المرئة
Age	:52 Y	Department	:Out Patient
Refereing Doc :		Registered by	:Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Left breast cancer.
Nature of Specimen: Slide revision.

Gross Examination:
One paraffin block labled by (P012-25) and one H&E slide were received.

Microscopic Examination:
* Histological type: Invasive duct carcinoma.
* Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+2) Grade II.
* DCIS: Focally positive.
* AngioInvasion: Could not be detected within the limits of biopsy received

Diagnosis:
Left breast mass, Slide revision:
Invasive duct carcinoma, Grade II with focal DCIS.
- For further IPT (ER, PR, Her2 and Ki67).

Signature
Prof. Dr. Lubna Omer
Dr. Menna Nabil

Verified by :
Dr.Heba Hasan

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مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

السيدات المصرية هي دايما مصر

MDT FORM 1/2/2025 GM

Name	عناق احمد السيد عيسوى	MRN	٢٠٢٥١٠٨
Age	52	ID	٢٧٠٢٠٠٤٠٢٢٦
Menopausal status		Phone	٠١٢٨١٤٨٤٠٦٢
NRS SCORE		Marital status	
BMI		Family history	
CO morbidity		REASON OF MDT	NEW

Clinical Data

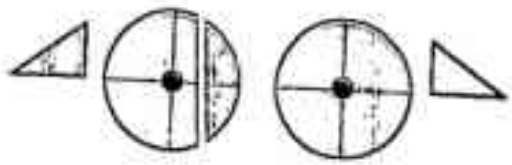
BX: IDC

ER:	PR:	HER 2:	KI67:
0	0	+3	

Sonomammography:

Mets Workup:

R L



TNM

- Mediastinal L.N
- Bilobar F.L

Recommendations:

MDT OUTCOME & PLAN

1/2/2025 → for - 1st line Taxol (herceptin IV (3C)
For Re evaluate.
- baseline CT chest, PA etcontra

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

جوستافروسي

مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

البنت المصرية هي صحة مصر

MDT FORM / /٢٠ G

Name	كفاني محمد السيد عويس	MRN	
		ID	
Age	52	Phone	
Menopausal status		Marital status	
NRS SCORE		Family history	
BMI		REASON OF MDT	
CO morbidity			

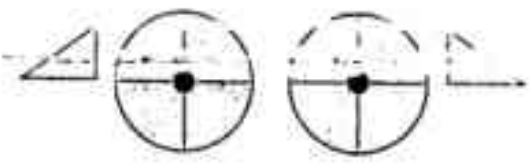
Clinical Data

BX: IDC CD

ER:	PR:	HER 2:	KI 67:
0	0	+3	

Sonomammography:

R L



Mets Workup: Metastatic Rt breast Ca to bone, liver, L.N. Received Taxol / Herceptin x20 week CT.CAP 21/5/20 → RD of multiple lesion

TNM

Recommendations: RD of axillary L.N. For MDT for next plan of Management

21/7/20

MDT OUTCOME & PLAN

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

GUSTAVE ROUSSY
INTERNATIONAL

Patient Name: Afaf Ahmed Al Sayed Elsawy	Date : 21/07/2025
Exam : Axillary u/s	Age : 53 Year(s)
Medical ID: 108/2025	Pat. ID: 96847

Soft tissue ultrasound examination of right axilla

- *Few enlarged axillary LNs at level I, the largest one is measuring 18x7mm showing relatively thickened cortex 3.5mm? reactionary for follow up*
- *Normal skin thickness and subcutaneous tissues*
- *Normal vascular structure*

- Signature Dr . Nermien Ramadan
- 21/07/2025 12:35 PM

Patient Name: Afaf Ahmed Al Sayed Eisawy	Date : 07/08/2025
Exam : Item 1	Age : 53 Year(s)
Medical ID: DAR ELSALAM HOSPITAL^BRAIN	Pat. ID: 96847

MRI EXAMINATION OF THE BRAIN

MR Technique:

- Axial T1, T2 and FLAIR WI's
- Sagittal T1 WI
- Coronal T2 WI
- Axial DWI and ADC
- Post contrast T1 series

MR findings:

- W dened cortical sulci, basal cisterns, cerebellar folia and extra-axial CSF spaces.
- Mild bilateral symmetrical dilatation of the supra-tentorial ventricular system
- Nc cerebral, cerebellar or brain stem areas of altered MR signal.
- Nc midline shift or deformity.
- No extra axial collections or masses.
- No cerebello pontine angle masses.
- No gross posterior fossa abnormalities.

Opinion:

- **Mild age matched brain involutional changes.**

Dr.Zeyad El-Sayed,FEBR

GUSTAVE ROUSSY INTERNATIONAL EGYPT

Patient Name: Afaf Ahmed Al Sayed Eisawy

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

Findings:

Compared to 21/5/2025

I Chest:-

- **Stationary course** of the:
  1. Left breast retroareolar irregular spiculated mass lesion seen measuring 2 X 1.28 cm compared to 4 X 5 cm seen reaching the nipple and thickened skin.
  2. Pathologically enlarged left axillary level I and II lymph nodes being subcm in current study.
- **Stationary course** of one of the right axillary level I lymph nodes reaching 9 X 12 mm.
- Non appreciated forenoted enlarged left lower deep cervical lymph nodes in current exam field.
- No sizable pulmonary nodules (apart from currently noted left basal 3 mm nodule), masses, areas of consolidation or cavitary lesions.
- **Stationary course** of the enlarged mediastinal lymph nodes being subcm.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.



II Abdomen & pelvis:

- The liver is of average size with regressive course of the bilobar hypodense hepatic focal lesions being hardly appreciated in current non contrast study.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

***Scanned skeleton:** more sclerosis L5 vertebral body tiny lytic lesion suggesting favorable response.

Stable L3 sclerotic lesion mostly bone island.

Opinion:

- **Stationary course since the last study.**

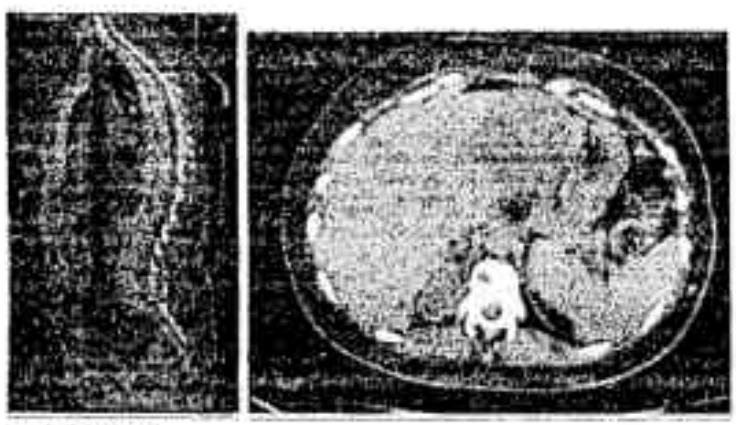
Date 12

Dr. AYA GABR

Name : Afaf El Sayed
Date : 20-7-2025

MULTI-SLICE NON CONTRAST CT SCAN OF THE ABDOMEN AND PELVIS: - PRIMARY REPORT

- ***Known case of right breast cancer. Compared to the previous study dated 5-3-2025, the current study revealed:***



Findings:

- The liver is still seen mildly enlarged (16 cm) with homogenous CT texturestill showing hypodense rather cystic focal lesion occupying segment II and III currently measures about 8x7.8x8.5 cm (compared to 8.2x7.5x7.5 cm), showing thin septations.
- No intra-hepatic or extra-hepatic biliary radicle dilatation.
- Normal size (11.5 cm) and parenchymal texture of the spleen with no sizable focal lesions seen. Still noted two splenules.
- Normal size, parenchymal thickness and density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas and both supra renal glands.
- Still noted multiple subcentemetric abdominal lymph nodes (para-aortic, mesenteric, right common iliac, left external iliac and bilateral inguinal).
- No ascites.
- Average filling of the urinary bladder with no definite stones or masses.
- No gross adnexal lesions.
- Still noted multiple axial and appendicular skeleton lytic lesions with no associated soft tissue component or structural vertebral collapse.

Ministry of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Afaf Ahmed Al Sayed Eisawy	Date : 21/05/2025
Exam : CT CAP(CT Chest Abd. Pelvis)	Age : 53 Year(s)
Medical ID: 108/2025	Pat. ID: 96847

NON CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

Findings:

Compared to 13/1/2025

I Chest:-

- **Regressive course** of the:
  1. Left breast retroareolar irregular spiculated mass lesion seen measuring 2 X 1.28 cm compared to 4 X 5 cm seen reaching the nipple and thickened skin.
  2. Pathologically enlarged left axillary level I and II lymph nodes being subcm in current study.
- **Increase in size** of one of the right axillary level I lymph nodes reaching 9 X 12 mm.
- Non appreciated forenoted enlarged left lower deep cervical lymph nodes in current exam field.
- No sizable pulmonary nodules (apart from currently noted left basal 3 mm nodule), masses, areas of consolidation or cavitary lesions.
- **Regressive course** of the enlarged mediastinal lymph nodes being subcm.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II Abdomen & pelvis:

- The liver is of average size with regressive course of the bilobar hypodense hepatic focal lesions being hardly appreciated in current non contrast study.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

***Scanned skeleton:** **more sclerosis** L5 vertebral body tiny lytic lesion suggesting favorable response.
Stable L3 sclerotic lesion mostly bone island.

Opinion:

- **Increase in size of one of the right axillary lymph nodes for sonographic assessment.**
- **Overall regressive course as described.**

Dr. Mohamed Mohsen, M.D.

Patient Name: Afaf Ahmed Al Sayed Eisawy	Date : 21/07/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 53 Year(s)
Medical ID: 108/2025 1720 1000	Pat. ID: 96847

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST PRIMARY REPORT

- ***Known case of right breast cancer. Compared to the previous study dated 5-3-2025. the current study revealed:***



Findings:

- Right breast operative bed still shows an ill-defined soft tissue density resting over the chest wall muscles measuring 3x1.5 cm for clinical and SM correlation.
- Still noted few subcentimetric left axillary lymph nodes.
- Still noted ubcentimetric left supra-clavicular lymph node.
- Still noted right lower lung lobe posterior segment tiny partially calcified pulmonary nodule about 4 mm.
- Still noted right upper and middle lung lobes subpleural reticulations and faint ground glass opacities, likely post radiation sequel.
- Left basal atelectatic bands are still noted.
- No sizable hilar or mediastinal lymph nodes regarding this non contrast study.
- Patent tracheo- bronchial tree.
- Average size of the heart.
- No evidence of pleural or pericardial effusion.

Patient Name: Afaf Ahmed Al Sayed Elsawy	Date : 21/07/2025
Exam : CT CAP+C (CT Chest Abc. Pelvis)	Age : 53 Year(s)
Medical ID: 108/2025 1720 1000	Pat. ID: 96847

OPINION:

- ***Stationary course of the disease process.***

Marina Essam, MSc

مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

MDT FORM 1/2/2025 GM

Name	عفاف احمد السيد عيسوى	MRN	C٤٥١٠٨
Age	52	ID	٢٧٠٤٠٠٠٤٢
Menopausal status		Phone	٠١٤٥١٤٥٤٦٢
NRS SCORE		Marital status	
BMI	[redacted]	Family history	
CO morbidity		REASON OF MDT	NEW

Clinical Data

BX: IDC

ER:	PR:	HER 2:	KI67:
C	0	+3	

Sonomammography:

Mets Workup:

R L

TNM
- Mediastinal L.N
- B.lobar F.L

Recommendations:

MDT OUTCOME & PLAN

1/2/2025 → Fit - 1st line Taxol (herceptin IV) (SC)
Then Letrozole.
- baseline CT chest, FA etcetera

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

جوستافروسي

مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

البنك المصرية هي صحة مصر

MDT FORM / /20 G

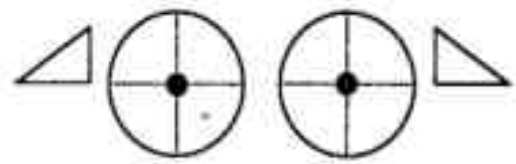
Name	دعاء أحمد السيد	MRN	10812025
		ID	
Age	52	Phone	
Menopausal status		Marital status	
NRS SCORE		Family history	
BMI		REASON OF MDT	
CO morbidity			

Clinical Data

BX: IDC GIII

ER:	PR:	HER 2:	KI67:
0/8	0/8	+3	

R L



Sonomammography:

Mets Workup: Mets breast Cancer to Liver & LN TNM
received 1st line taxol weekly + Herceptin
last CTCAP → SD. (Cycle 4 to be)

Recommendations: for MDT. i.e. BS Cycle 4
5-2025
Dr 31/7/2025. Consider 2nd line Xeloda-Tykerb

MDT OUTCOME & PLAN

SIGNATURE					
MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

١ - صدور محضر كامل إداري العهد سوكور
٢ - هيئة تراثية للروحين وتسمى هذا (٢)
٣ تنبيه دراسي لمن خوف ٢٠
٤ أيهما مرفق





٢٧٩٠٥٢٩٠١٠٢٤٦٩ ٢٠١٦/٠٩

دبلوم تجاره

الثنى مسلمة متزوجة

عصام احمد محمود

البطاقة سارية حتى ٢٠٢٣/٠٩/٠٨

٩





جوستافروسي

إسم المريض ثلاثي: .....
رقم الملف: .....

تقريـر طبـي

السيد/.....

تحية طيبة وبعد،،

إيماء للطلب المقدم من ..... بشان تقرير طبي للمريض: ضعاف الم

بقسم.....رقم طبي.....

نتشرف بأن نوضح فيما يلي تقريرا عن حالته :

تاريخ وساعة الدخول.....تاريخ وساعة الخروج.....

وبعد الكشف والفحوصات والمتابعة وجد انه:

Pt 54 y.o.
A Case of ABC to bone & LN
recived t.xt & Herceptin → D.P
Yelda + tykerb → D.P
by Bone Sol.
for possible of IDM.

تحريرا في

الوقت: .....

التاريخ : ٢٠١٦/١٢/٢٦

الطبيب المعالج

.....

يعتمد